RSA

Interface and Functionality Guide

Roles, wards, reports, conversations,
diary, shift handovers and document review

Understanding the software we have built
This guide collects and updates the explanations requested during the conversation. Screenshots come from the actual Flutter frontend using fictional data. Available functions are distinguished from services still requiring backend integration.

29 October 2026
Code baseline: `6282ddf`
Informational document; not a clinical protocol or a compliance certification.

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1 How to read this guide

RSA organizes ward work by linking observations, communication and the transfer of information between shifts. The same fall can be an entry in the diary, an item to handle in the Inbox, the context of a conversation and a source for a handover. These are not four separate falls: they are four ways of working with the same event.

This guide describes Flutter frontend version `6282ddf`, reviewed on 29 October 2026. Button names match the English labels shown in the images. Screenshots use fictional users, residents and data, with a controlled clock. Close-ups crop the same original screenshots rather than reconstructing imaginary interfaces.

> **Remember.** The frontend already provides interactions, local permissions and persistence, but the production backend is not connected yet. Real login, push delivery, external AI/OCR, encrypted archiving and protected auditing still need integration. A visible feature does not prove that its external service is operational.

1.1 Distinctions that prevent common misunderstandings

Action or section	Meaning
Reading an event	Consulting its content; not assuming responsibility.
Take charge	Explicitly assuming responsibility for handling an event, when eligible.
Close	Recording the outcome and closing the event; requires the appropriate responsibility.
Open chat	Opening the linked conversation and adding it to your list; not taking charge.
Add correction	Adding a tracked note to the original report; not silently rewriting it.
Sign handover	Signing a specific handover version as an authorized coordinator.
Acknowledge version	Explicitly confirming receipt of that version; not closing its events.
Document review	Comparing source and transcription, correcting and confirming the text; not automatically validating clinical content.

2 Users, roles and wards

2.1 The site is not limited to Oak Ward

Oak Ward is the ward used in Emily Carter's initial data. The model also contains Willow Ward and can support other wards. Each user has assignments combining a ward and a role; they may have several roles in one ward or assignments in different wards.

Current workspace/Current ward identifies the active ward. Clinical pages filter information by that context and the user's permissions. Authorization in ward A does not authorize access to ward B. Multiple roles within one ward combine their capabilities; a role assigned elsewhere does not automatically carry over to the active ward.

The Oak Ward card at the bottom left is therefore a context indicator. The promotional sentence "A connected shift, a shared picture" has been removed: there is no mandatory image to put in its place. Connected badges and demonstration references are also absent from the normal operational interface.

2.2 Does an interface exist for every role?

Yes. The code contains the capability model and appropriate interfaces for the seven roles below. Some share the same page with different actions; an identical page does not need to be duplicated for every profession. Specialist workspaces are separate.

Role	Available frontend functions
Nurse	Home and attendance, reports, conversations, diary, document review, handover contributions and acknowledgement of assigned versions. May take charge when on duty and eligible, and close events for which they are responsible. Cannot sign the coordinator's handover merely by being a nurse.
Healthcare assistant	Reports and observations, communication, ward documentation, contributions and acknowledgement of assigned handovers. Does not automatically acquire nursing responsibility or medical-validation authority.
Technician	Reports, including faults and safety issues, communication, authorized documentation, contributions and acknowledgements. The technician role alone grants neither nursing take-charge nor physician-validation rights.
Physician	Clinical workspace and Clinical review; may review/edit a draft's clinical content and validate it when required. Cannot sign as coordinator without also holding that role.
Coordinator	Care workspace and Coordination. Supervises staffing, availability, skills and open work; prepares/reviews/signs handovers, selects the incoming team and opens new versions. Cannot create accounts or freely assign global permissions.
Management	Management workspace with aggregate indicators and filters for authorized wards. No automatic access to clinical details. May maintain an authorized ward's display name.
Administrator	Administration workspace for existing users' roles/wards, routing timings, integration status and audit metadata. No implicit clinical access. New accounts are inserted manually in the database.

The coordinator is comparable to the ward group's operational lead: they supervise work and shift handover. Group membership follows authorized assignments. This is not the technical administrator of the entire application.

2.3 Why does login currently always open Emily's account?

As requested, Login enters the local Emily Carter, Nurse, Oak Ward account without credentials. The normal interface has no demonstration selector for impersonating other people. Once the backend is connected, the authenticated identity will determine the roles and workspaces to display. Other roles were documented using test identities only in the capture tool, without adding controls to the public UI.

3 Access, Home and navigation

3.1 Login first, with no self-registration

Opening the site displays Login. There is no self-registration: users are inserted manually in the database. Any old registration URL redirects to Login.

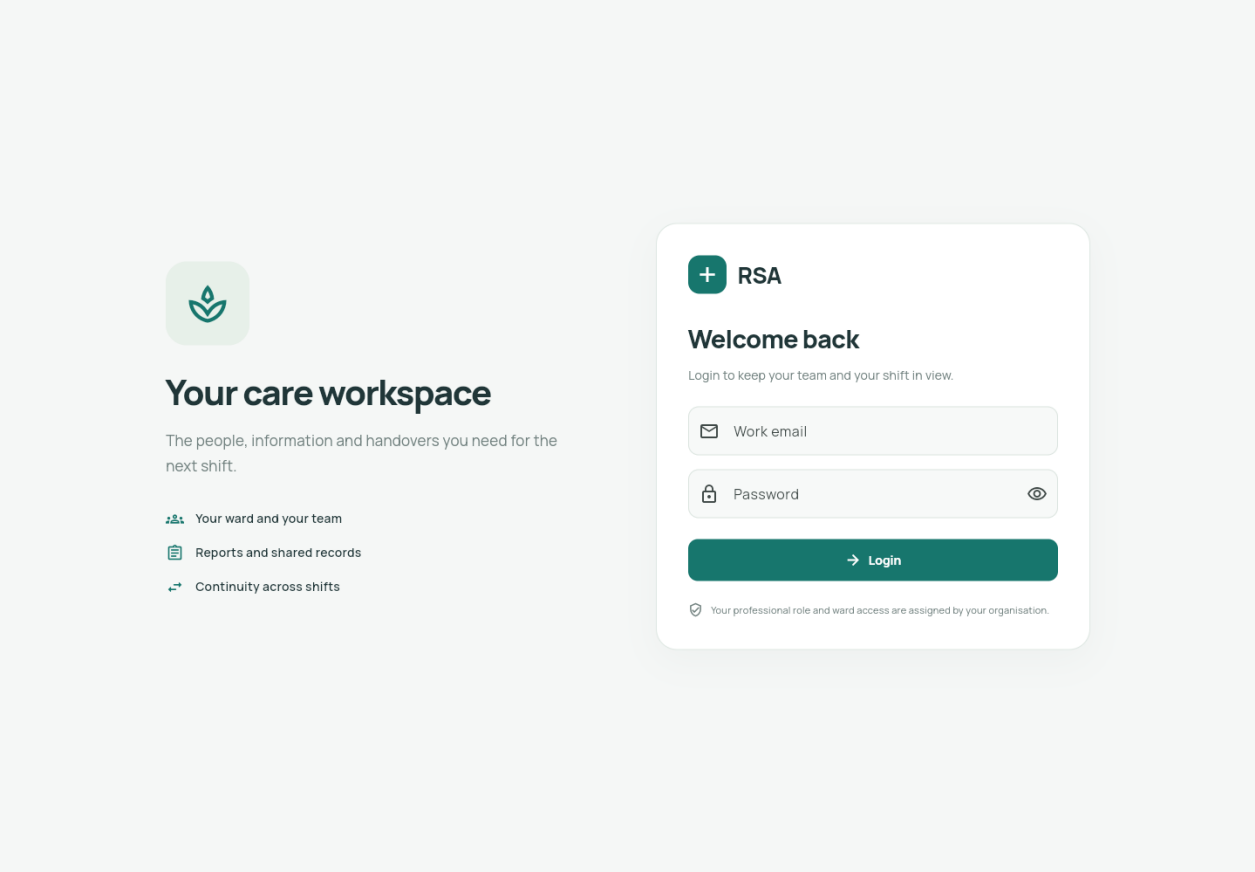


Figure 1: Initial page: login without public registration.

For now, you can explore by pressing Login without entering data. Emily's Home opens. This shortcut supports frontend integration and does not verify a person's identity. The current session is held in memory: a full page reload returns to Login.

3.2 Sidebar and profile

Overview is Home; Chat opens Conversations; Inbox is the operational notification list; Ward diary is the ward chronology; Shift handover contains shift-transfer documents; My shift concerns attendance; Document review supports transcription review. Clinical review, Coordination, Management and Administration appear according to role.

Navigation counters indicate relevant/unread items, not the absolute total of every report in the facility. Report event remains a shortcut to a new report. The photo/initials and name at the top right open Account settings. The other settings icon to the left of the profile was removed because it opened the same page.

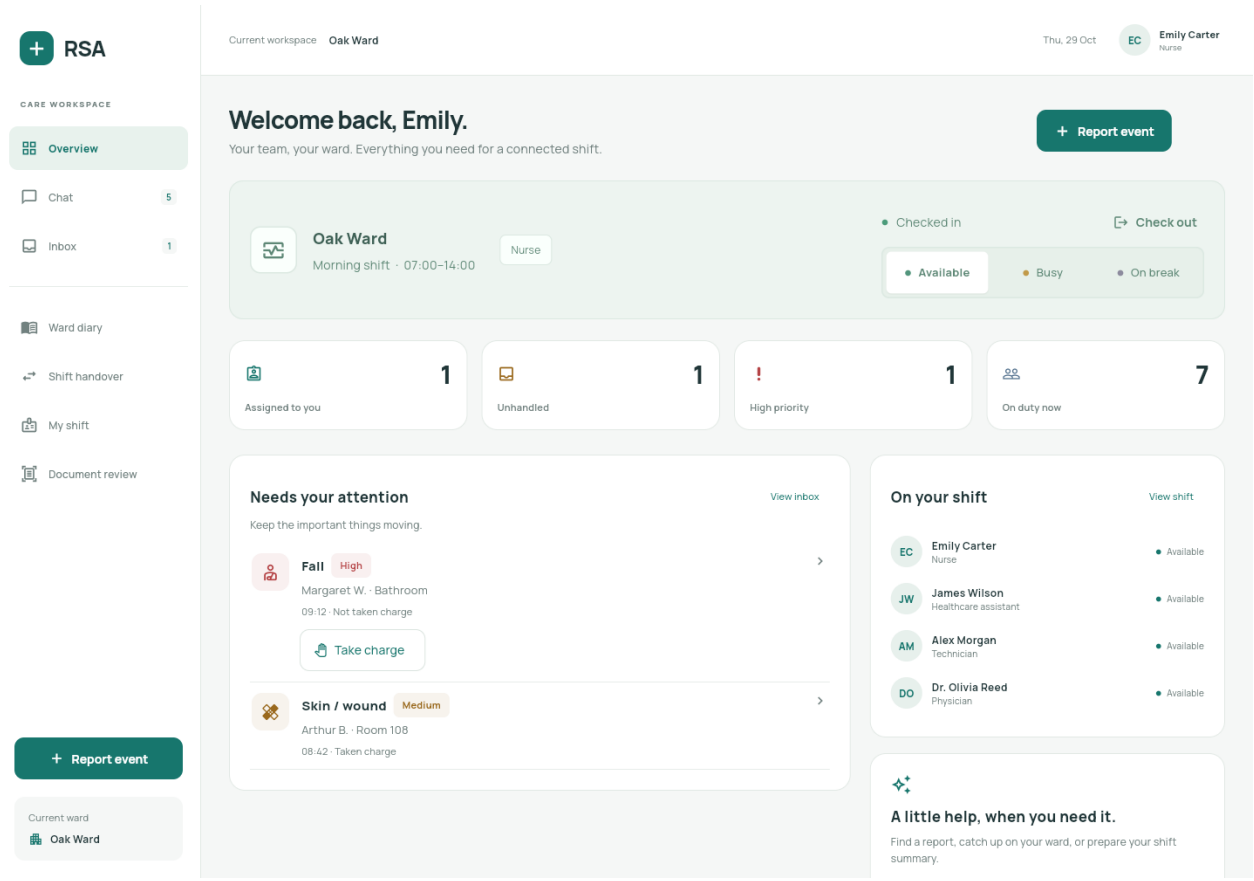


Figure 2: Nurse Home and the shared navigation structure.

3.3 The check-in and check-out card

The left side identifies ward, shift, hours and role. The right side distinguishes attendance from availability. Check in records the start of attendance; Check out ends it. Available, Busy and On break describe working status and affect report recipients according to routing rules.

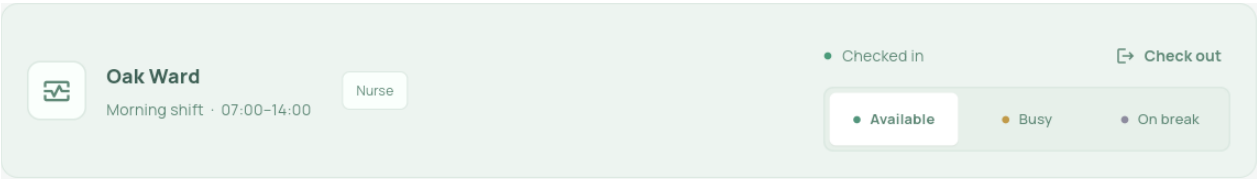


Figure 3: Close-up: ward/shift, attendance and availability in one card.

Being logged in does not mean being on duty. Recorded attendance is not physical location tracking and does not use GPS. While a change is being saved, the control prevents duplicate submissions. The current model does not replace full future rostering or independent simultaneous attendance records per ward.

4 Reporting and handling an event

4.1 Completing a report

From Report event, select the type, priority, resident or Other, location and occurrence time, then add notes or attachments if useful. The type suggests a priority, which you can change before

submission.

Type	Example content	Suggested priority
Fall	A fall or a resident found on the floor	High
Health	A change in health condition	High
Wandering	Leaving the ward or a missing person	High
Agitation	Agitation or aggressive behavior	Medium
Refusal	Refusal of food, drink or medication	Medium
Skin / wound	A wound, redness or skin change	Medium
Safety	A fault, equipment issue or environmental hazard	Medium
Hygiene	Hygiene assistance or incontinence	Low

This is the project's initial classification and must be validated against ward procedures. The application does not replace immediate care actions required in an emergency.

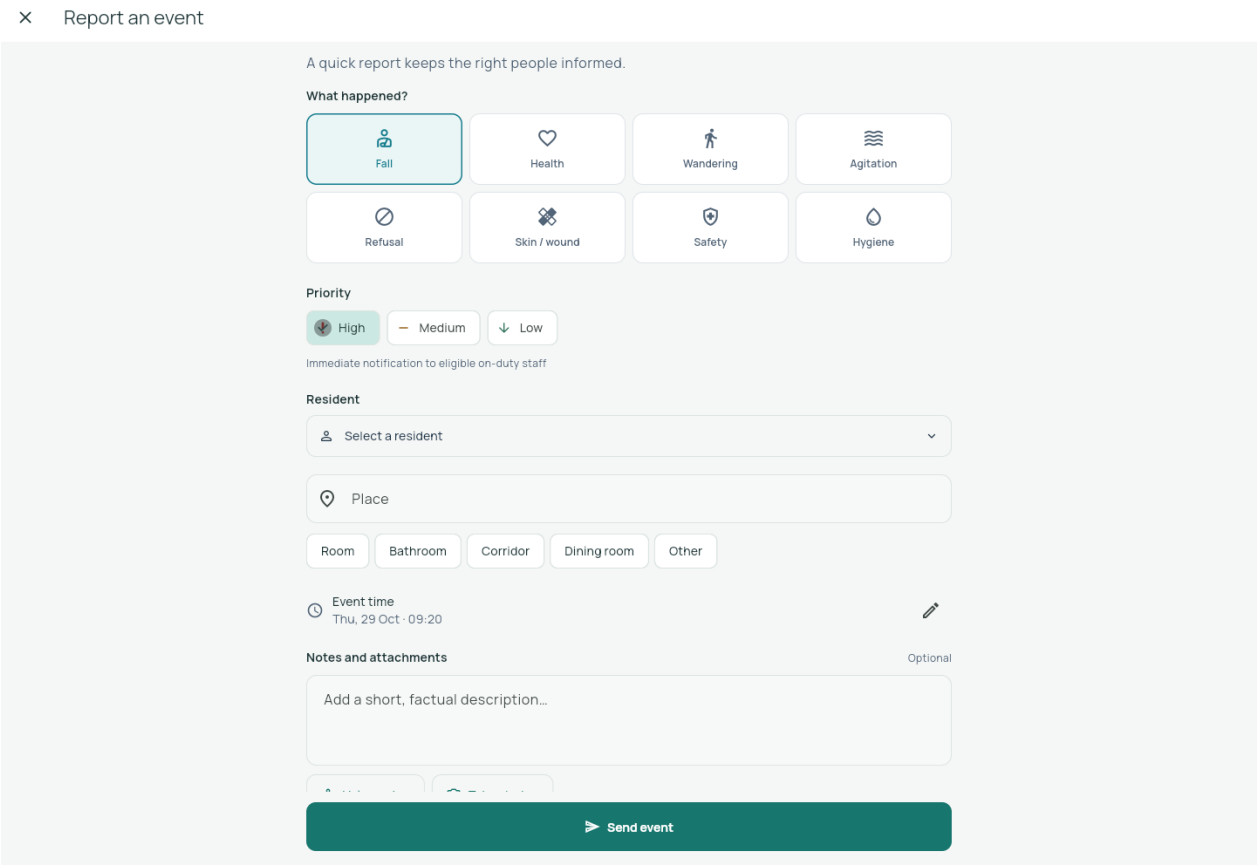


Figure 4: Event types and priority selection in the report form.

The resident picker searches the active ward. Other covers people or situations without an associated resident, such as an environmental issue. Selecting a resident suggests their room; you can replace it with a common place or free text. The RASD's actual staff-to-resident allocation per shift is not connected yet: the current restriction is by ward.

Occurred is the event's occurrence time and can be adjusted for an earlier observation. Registered is the initial recording time; a delayed submission may also have a synchronization time. Times

are managed as unambiguous instants and displayed in Europe/Rome, including daylight-saving changes.

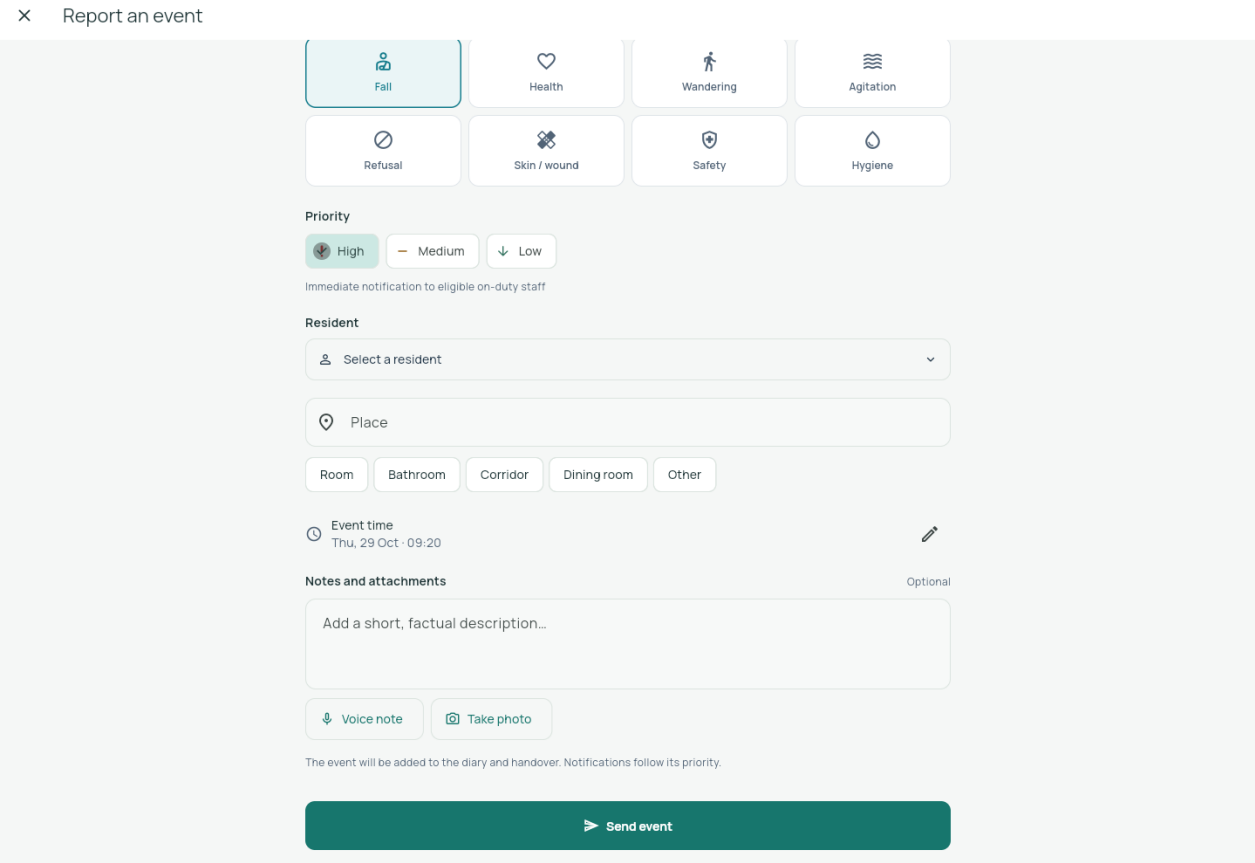


Figure 5: Location, occurrence time, notes and optional evidence.

Voice note records audio; it is not yet automatic speech transcription. Take photo/capture requires device permissions. Attachments can be checked before submission. The frontend handles drafts, a queue and retries without local duplication; real server submission will be connected through the backend. Without connectivity, a queued item does not mean a colleague has already been alerted.

4.2 Who receives the report?

Local eligibility uses ward, attendance, availability, take-charge capability and nursing skills. In the current configuration, High events involve eligible nursing/coordinating staff even when Busy, but exclude staff on break; Medium events select Available nursing staff. Low events feed the diary and handover without an immediate alert.

Local reminder/escalation deadlines are not an approved clinical protocol. Real audible/silent device pushes and a backend scheduler are not connected yet. Even after integration, push dispatch, reading and taking charge will remain separate actions.

4.3 Sections of the event detail page

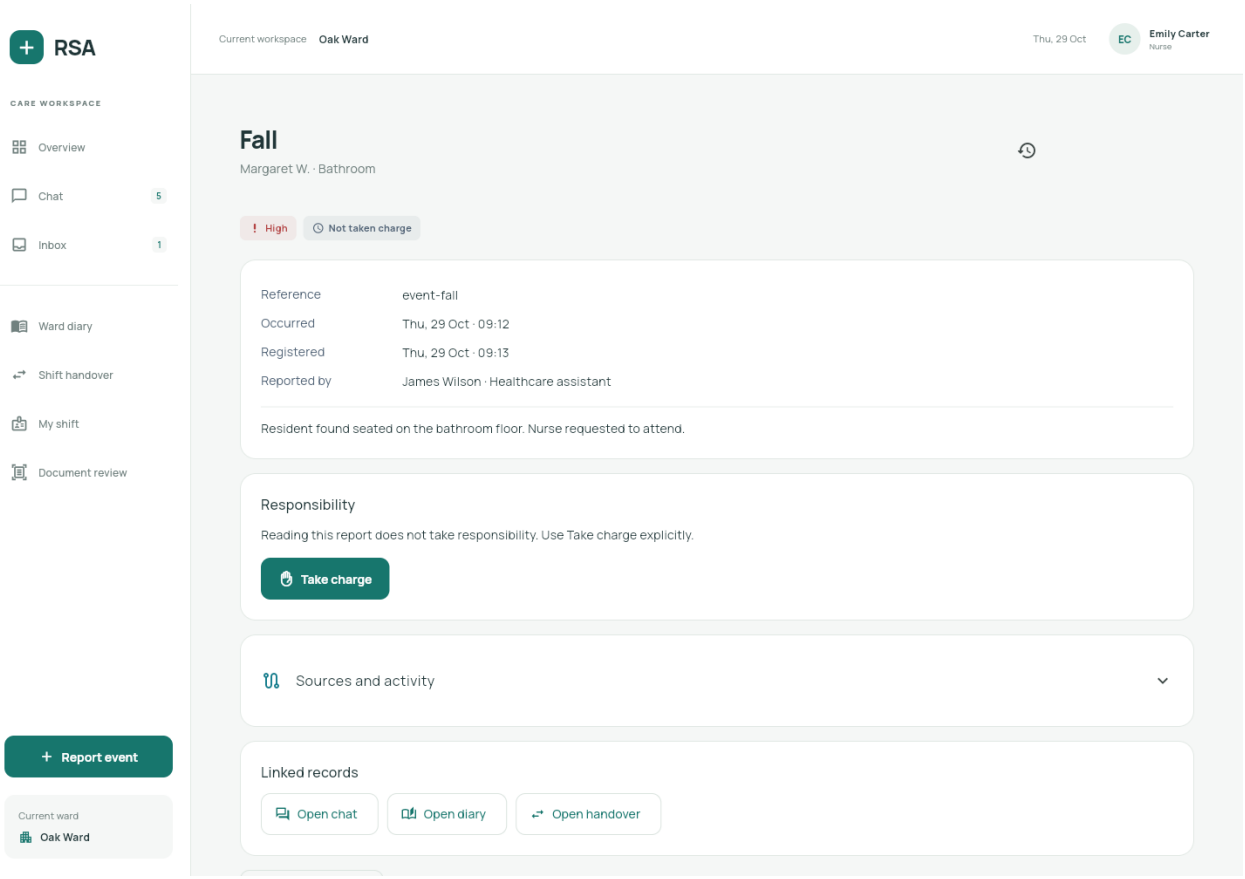


Figure 6: Event page: original information, priority/state and responsibility.

Section / field	Purpose
Title and subtitle	Event type, resident/subject and location.
High / Medium / Low	Priority, expressed by a text label as well as color.
Not taken charge / Taken charge / Closed	Handling state, separate from priority. A Medium event may be taken charge of but still open.
Reference	Record identifier; not a diagnosis or chat number.
Occurred	When the event happened.
Registered	When it was initially recorded.
Reported by	Report author and role.
Text / attachments	Original observation and associated evidence.
Responsibility	Who handles the event and explicit take-charge/closure actions. Reading does not take charge.
Sources and activity	Rule-selected recipients and notification/escalation information; for Low events, explains the absence of an immediate alert.
Linked records	Open chat, Open diary and Open handover links.
Add correction	Adds a tracked text correction.
Version history	Original text and corrections, with author and date/time.

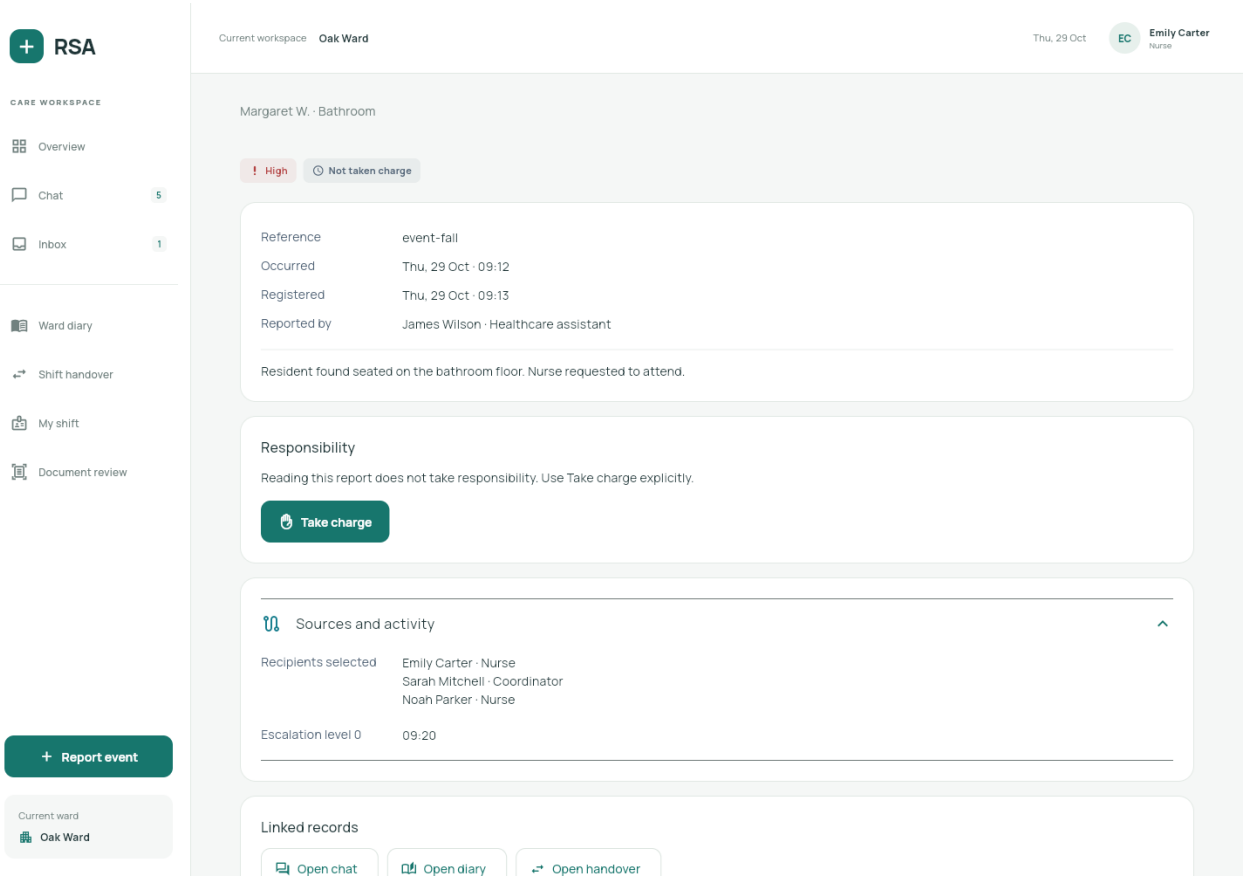


Figure 7: Expanded Sources and activity and linked-record navigation.

> **Remember.** In the current code, the time beside "Escalation level N" is the notification creation time, not the exact transition time to that level. Do not use it to reconstruct when escalation happened. The panel also uses notifications visible to your account: if you are not a recipient, an empty list does not prove nobody else was alerted. These are recorded completion items.

4.4 Take charge and Close

Take charge is an explicit action available only when role and attendance permit nursing responsibility. It records who assumes responsibility and when. The current model permits one active responsible person per event. Reading the report, discussing it in chat or acknowledging a handover does not make someone responsible.

Close records an outcome and closes the event through its eligible responsible user. Handover does not close events automatically. Delegation, reassignment and exceptional closure must be agreed with the ward before the final backend is implemented.

4.5 Add correction: what changes if it does not edit the report?

A correction adds later information while retaining the original report. For example: "Clarification after inspection: the correct location is the bathroom in room 104." The note can explain a correction, but this command does not currently rewrite the record's structured fields. Its purpose is to distinguish the initial observation from what was clarified later.

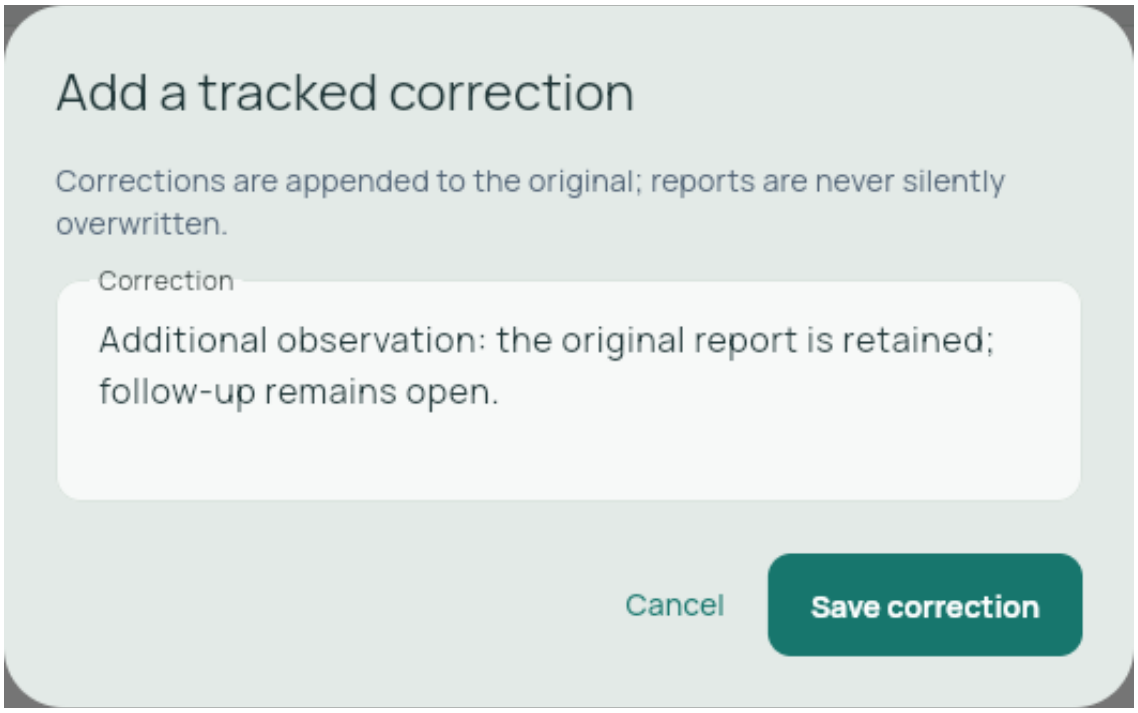


Figure 8: Correction dialog: added text, original retained.

4.6 Event Version history

History shows the initial version and later corrections, each with author, date and content. If an author is no longer within the accessible scope, their identifier may appear instead of their profile: out-of-ward profile access is not forced. Complete historical name/role snapshots are not yet immutable in every record. This history does not automatically list everyone who read the report and is not the complete technical access audit.

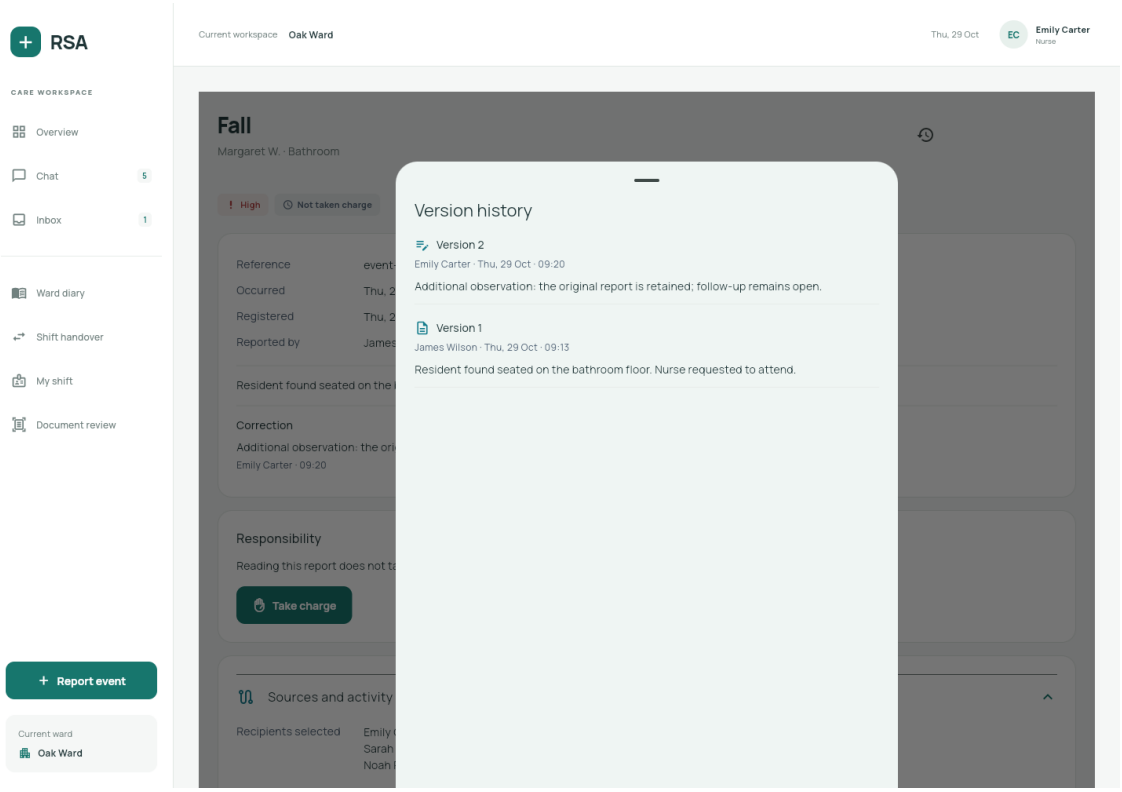


Figure 9: Event history after a correction has been added.

The red `_dependents.isEmpty` screen previously reported was a Flutter dialog-lifecycle defect, not a rule forbidding corrections. The current dialog retains its own controller until it closes. Any recurrence should be treated as a technical defect, not as an intended rejection of all corrections.

5 Inbox and Ward diary: differences and visibility

5.1 My items does not mean “reports I created”

The Inbox starts from notifications visible in the active ward. For nurses, these are notifications addressed to them; for coordinators, they are the ward’s notifications. Filters operate on this base set.

Filter	Actual content
My items	Items received by me or taken charge of by me within the visible-notification set. May include closed items. Authorship alone is insufficient.
Ward	Non-closed items from visible notifications. For coordinators it covers open ward notification work; for nurses it remains limited to their visible notifications.
Unhandled	Visible items not yet taken charge of.
Closed	Visible items explicitly closed.

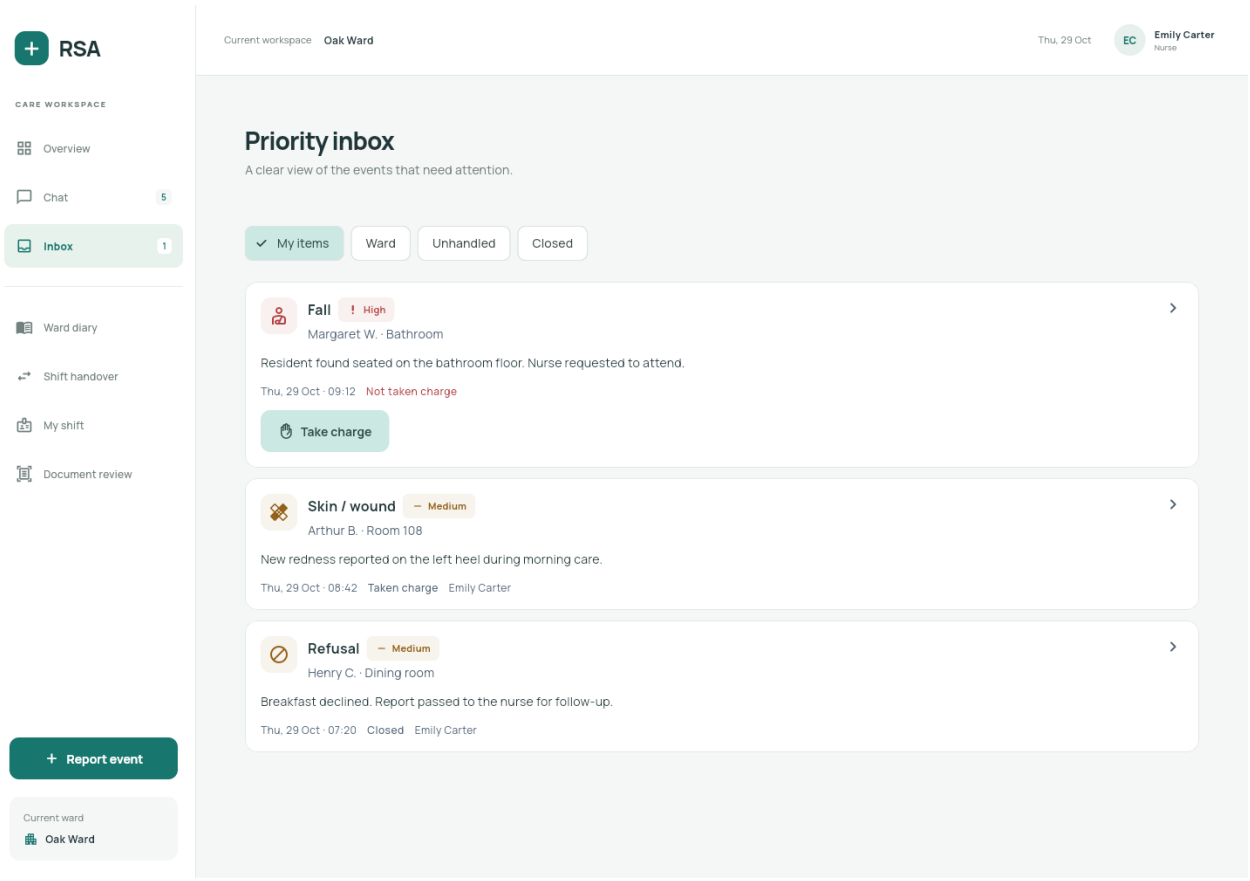


Figure 10: The nurse's My items: an operational personal list, not an authorship list.

For example, James reports a fall and Emily receives its notification. The report may appear in Emily's My items even though James is the author. A report created by Emily but not addressed to her does not enter her list solely because she authored it.

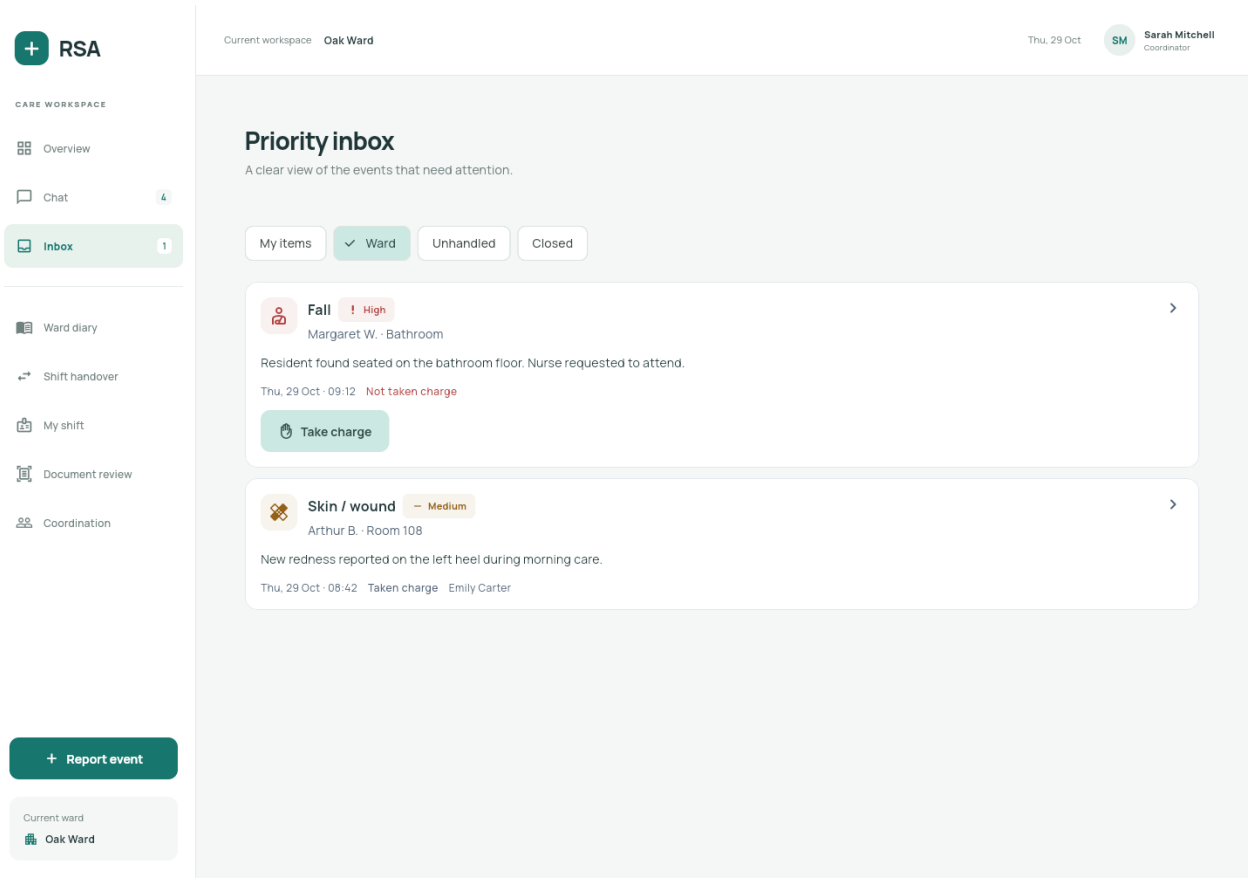


Figure 11: Coordinator Inbox: broader supervision of notification work in the active ward.

5.2 The diary is shared within the ward

Ward diary is the authorized ward's event chronology, including Low and Closed events. Search is accompanied by filters for local date, resident, type, priority and handling state. Users with the same permissions in the same ward read the same records. Changing ward or capabilities changes the accessible set; there is no unrestricted facility-wide access.

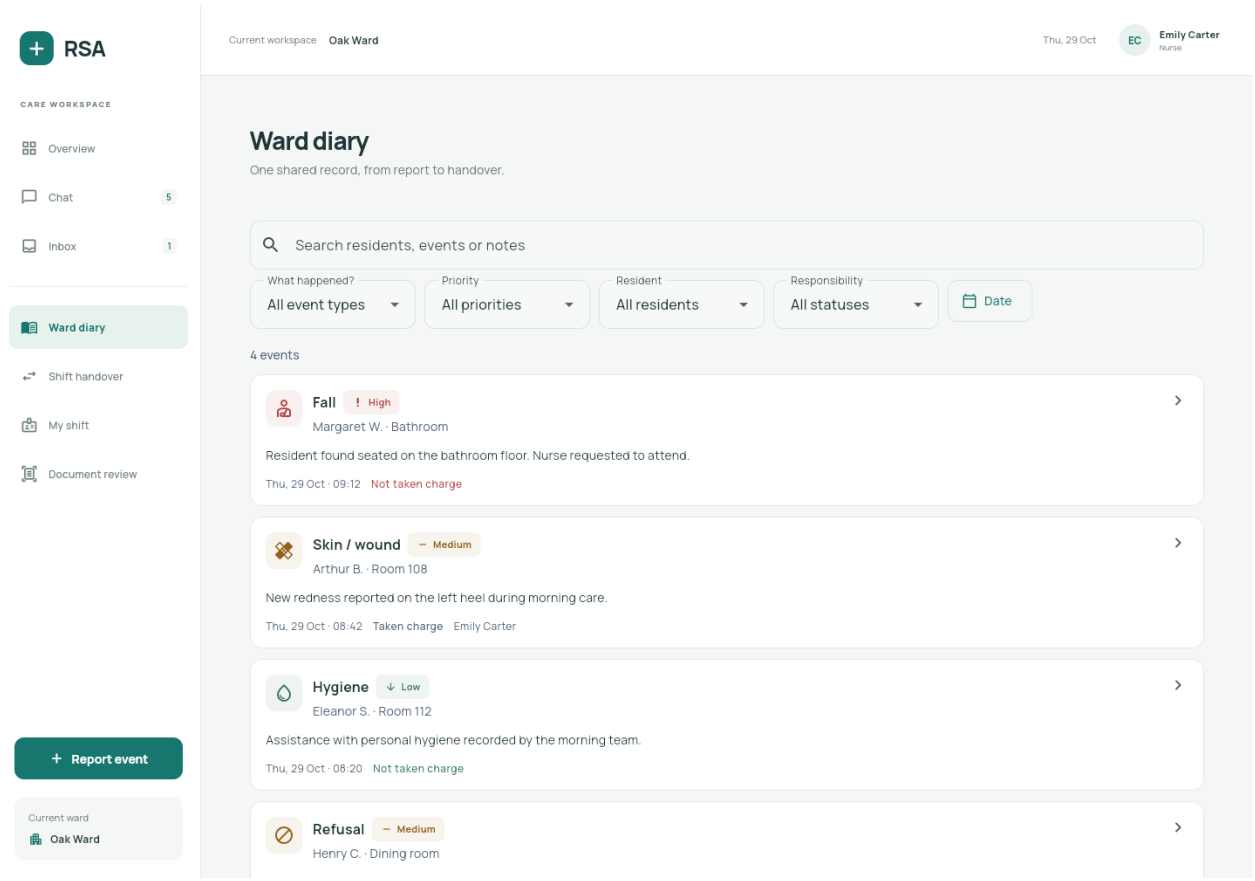


Figure 12: Ward diary with chronology, search and filters.

Inbox highlights work to receive/handle according to recipients and state; the diary preserves what happened in chronological order. Low-priority hygiene work may appear in the diary without an immediate Inbox alert. A Low event's generic Not taken charge label does not prove a notification exists.

6 Conversations and RSA Assistant

6.1 Why is there a chat linked to Fall or Refusal?

It keeps questions, clarifications and coordination close to the event they concern. The initial card summarizes context and View linked event returns to the official report. Staff can discuss a fall without losing its source in a general conversation.

Chat does not replace the report, is not its correction history and does not automatically take charge. A chat message does not become an official report correction or signed handover merely because it was sent.

6.2 Not every report chat appears in everyone's list

As requested, event chats are no longer all listed automatically. A user opens the report and selects Open chat; that conversation then appears in their list. Opening is personal: Emily opening the fall chat does not automatically add it to James's list.

This controls list visibility, not privacy exclusively for Emily. Authorized ward members with chat permission can access the event conversation even if they were not notification recipients. Other wards or roles without clinical/chat capabilities do not automatically acquire access.

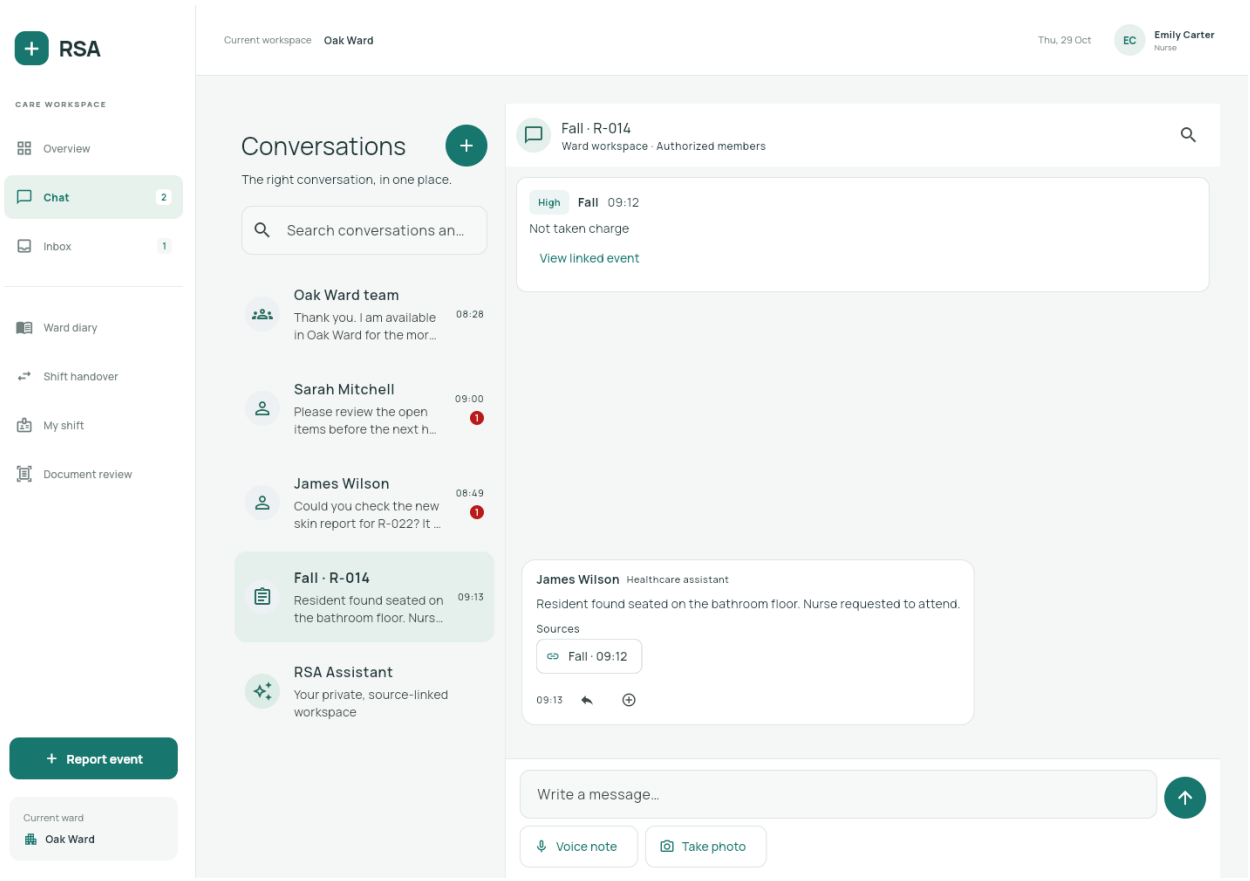


Figure 13: Event conversation after Open chat: context, source link and messages.

6.3 Conversation types and the plus button

Type	Who can consult it
Ward group	Authorized chat members of that ward.
Event chat	Authorized members of the event's ward; appears in the personal list after explicit opening. Not limited to push recipients.
Direct chat	Conversation participants; another ward colleague cannot view it simply by belonging to the ward.
RSA Assistant	The individual owner of the private assistant conversation.

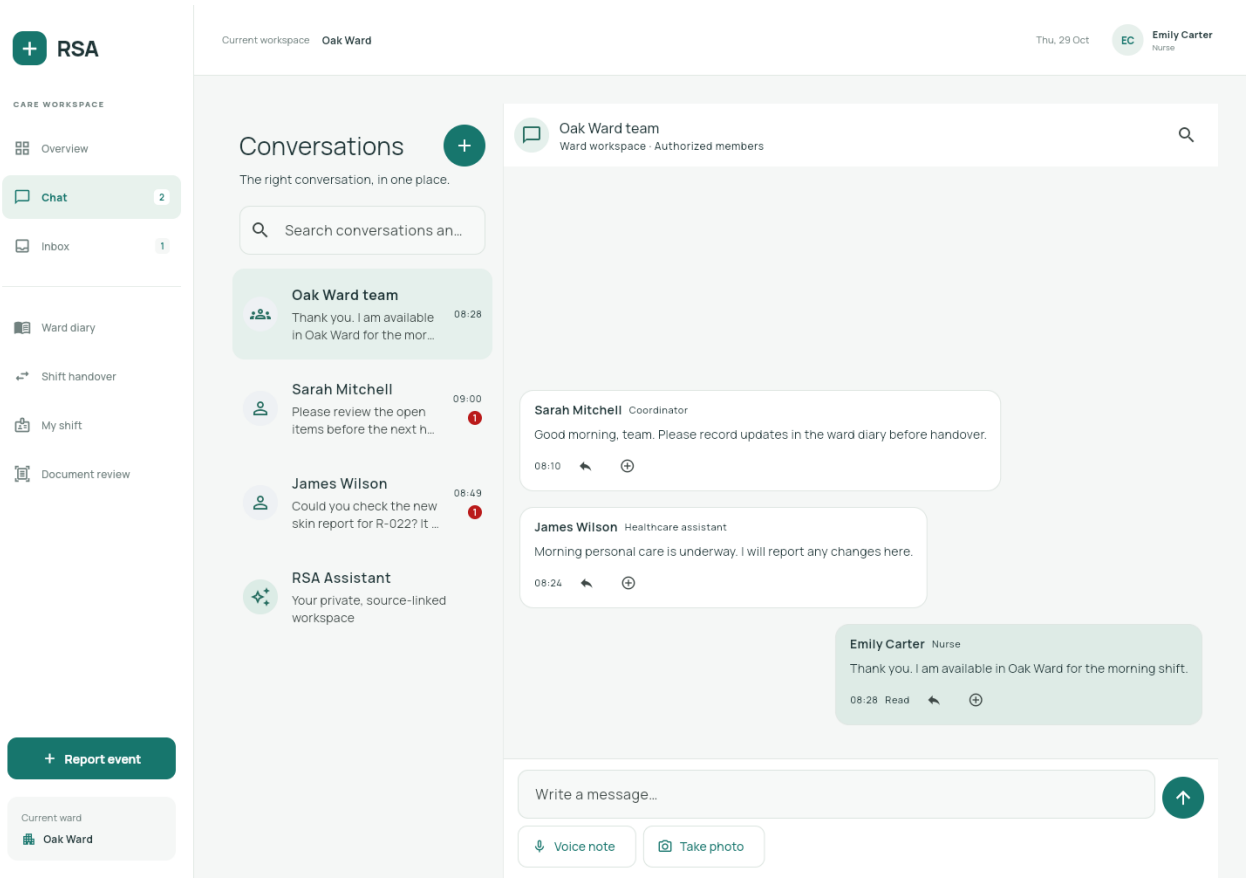


Figure 14: Conversations, with search and the new-conversation button.

Team chat was renamed Conversations. The + button opens colleague search within the active ward: select a person to start a direct conversation. An existing conversation with that person is reused. Other-ward users and users without chat permission are excluded; being off duty does not prevent someone from being an authorized contact.

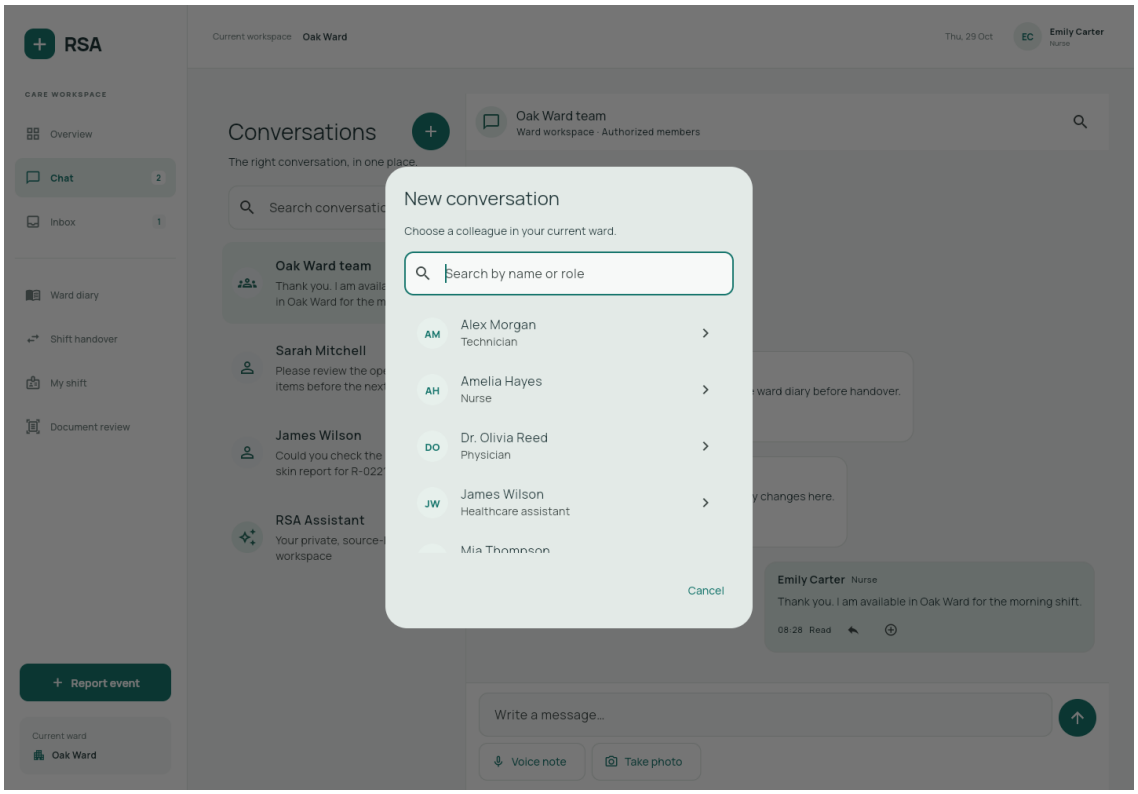


Figure 15: New direct conversation with an authorized colleague.

6.4 Messages, replies, attachments and sources

Bubbles show author, role, content and time/state where available. Replies retain context; voice/photo controls manage supported attachments. Sources references open authorized sources. Search, drafts and unread indicators assist consultation.

Converting a message to a report opens a form for review and submission: the message alone is not an official record. First opening uses a stable transition; the problematic initial scrolling effect was removed. Local delivery state will need connection to real backend confirmation.

6.5 What RSA Assistant actually does today

The assistant helps find and summarize facts in the authorized context, linking supporting reports. Its conversation is private to the individual user. It cannot query unauthorized wards, close events, take responsibility or sign documents on staff's behalf.

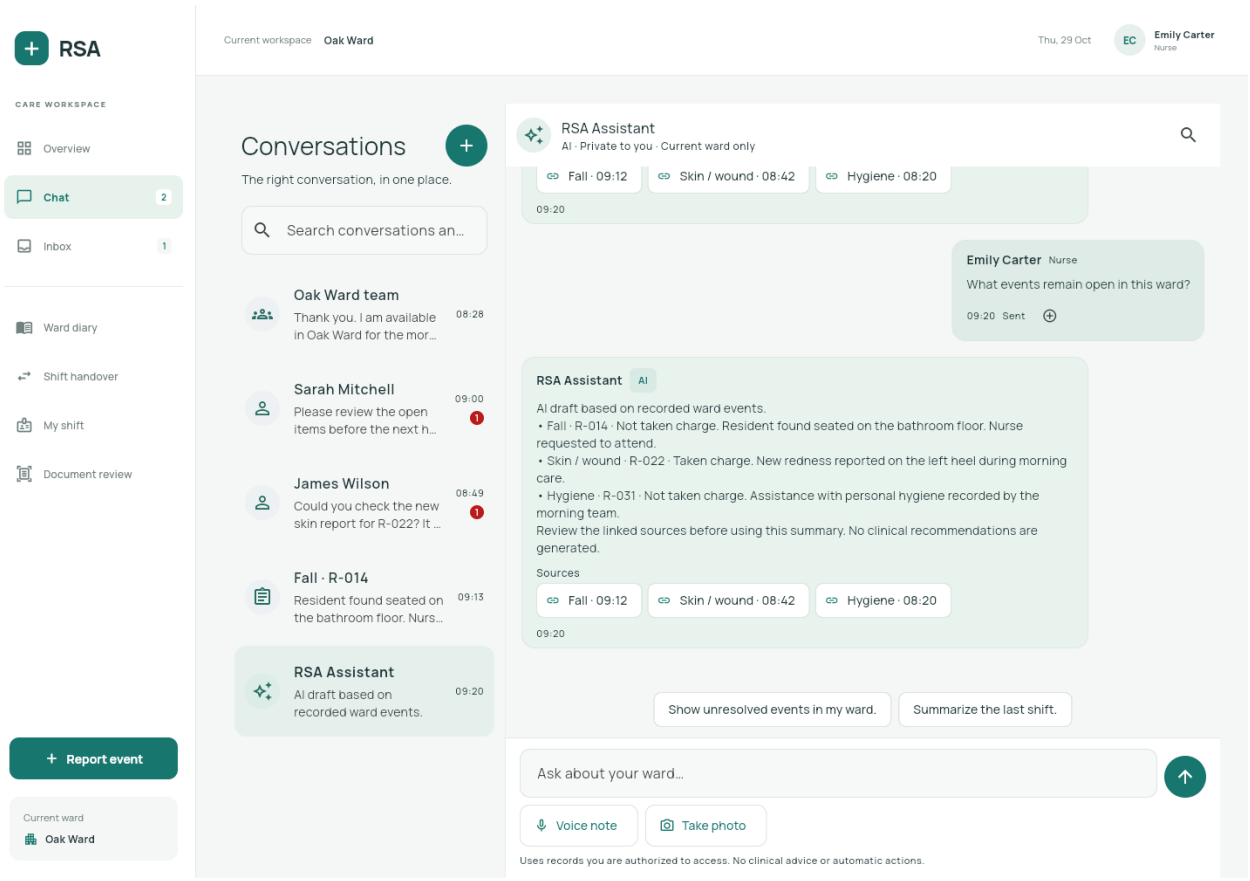


Figure 16: RSA Assistant answer with links to accessible sources.

Current answers are generated by local source-based logic; an external AI model is not connected yet. Future integration must preserve these permissions and human oversight. It is not an autonomous diagnostic or treatment-advice tool.

7 Shift handover: passing information to the next team

7.1 Its purpose and how it differs from the diary

A shift handover brings together important facts, work still open and information the incoming team needs. The diary preserves the chronology; the handover selects and organizes sources for a specific shift transfer and records review, signature and receipt of an exact version.

The page is tied to the selected ward, date and shift. The current model uses Europe/Rome and these hours: Morning 07:00–14:00, Afternoon 14:00–21:00 and Night 21:00–07:00. The night shift crosses midnight and the calculation accounts for daylight saving. The ward must approve the definitive source windows, late-event handling and carry-forward of unresolved work.

7.2 Page areas, from top to bottom

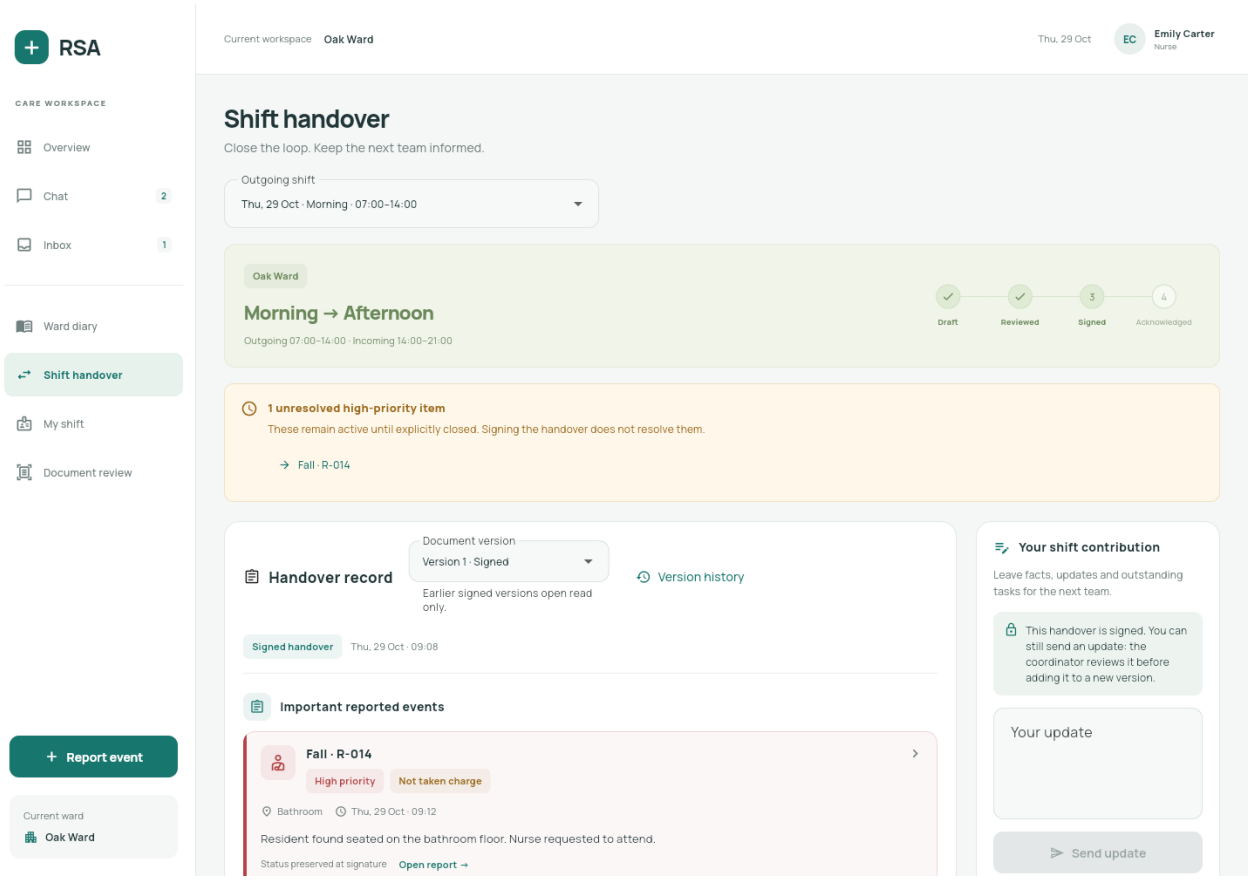


Figure 17: Handover page: shift context, workflow state, warnings and working columns.

Area	What it indicates or enables
Shift handover heading	Identifies the function and offers the appropriate role/state action: prepare a draft or open an amendment.
Outgoing shift	Selects the handover for a ward date/shift. Does not change each report's responsible person.
Morning → Afternoon banner	Shows ward, outgoing/incoming shifts and their hours.

Area	What it indicates or enables
Draft, Reviewed, Signed, Acknowledged	Shows document progress: preparation, review, signature and receipt. Clinical validation is an additional condition when configured.
Amber warning	Highlights High reports still open. Signing does not resolve them: they must be handled explicitly in event details.
Handover record	Content of the displayed version, with organized events, narrative and source links.
Document version	Selects the current version or an earlier signed version. Earlier versions open read-only.
Your shift contribution	Lets staff submit facts or matters for the next team. After signature, sends separate updates for review.
Incoming team	Lists assigned recipients and their acknowledgement state.
Version history	Shows versions, status, signatures, receipts and activity; this is not just the fall report's history.

7.3 Handover record: understanding the colored cards

Events appear as cards with a type icon, priority, handling state, location/time and relevant text. Red, amber and green aid scanning; High/Medium/Low labels and state text retain meaning without color. Open report opens the source.

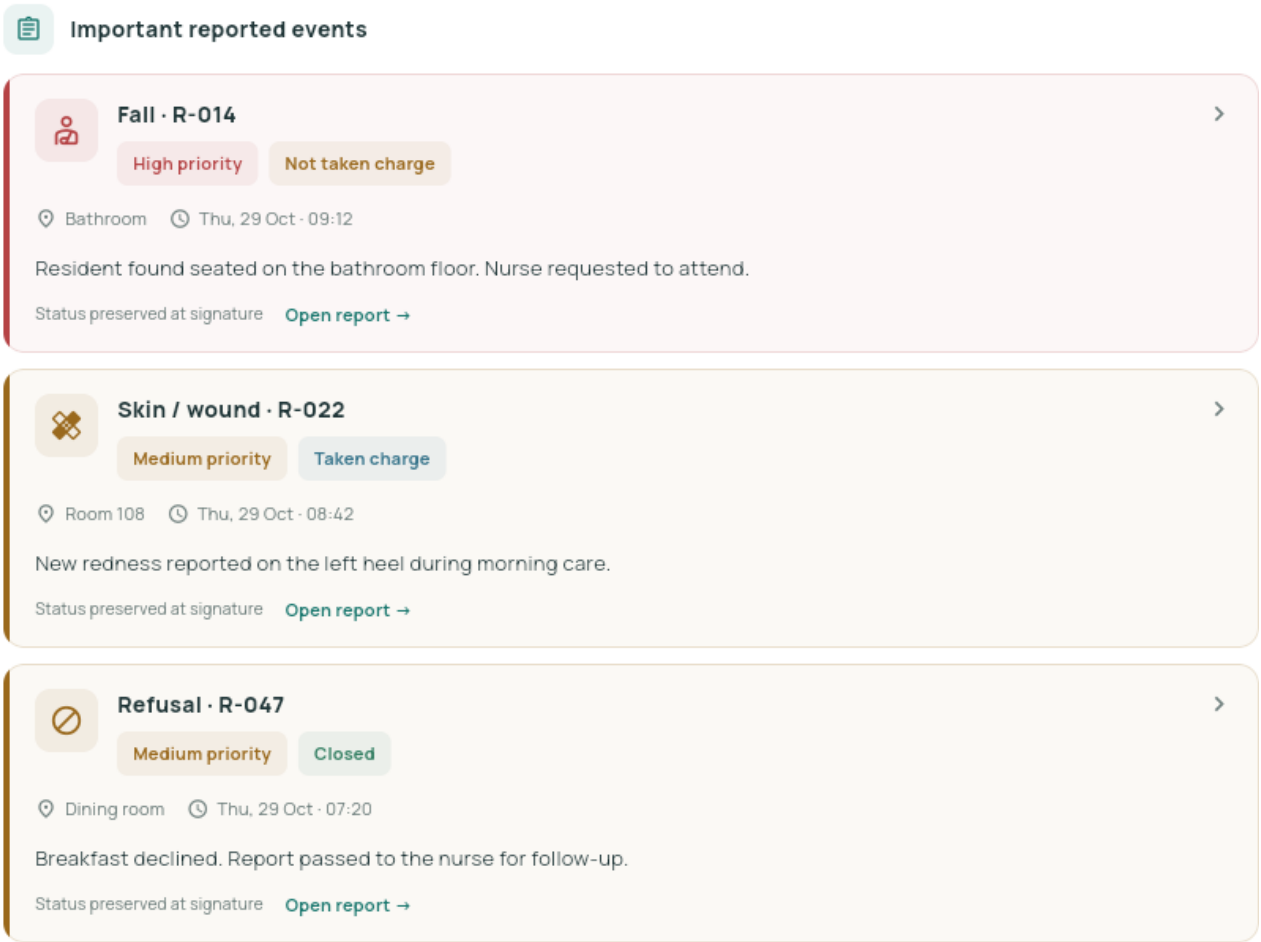


Figure 18: Card close-up: priority and handling are separate labels. State retained at signature is identified.

Content group	Meaning
Important reported events	Significant facts to communicate, including important events recently closed.
Open follow-ups	Work not yet closed. A Taken charge report may remain here: someone is handling it, but it is not resolved.
Unresolved notifications	Relevant unresolved notifications with priority/state.
Items to pass to the next shift	Staff contributions and matters to continue, including low-priority work.
Staff contributions	Source notes with author/time; not automatically signatures.
Linked source reports	Links to the reports behind the narrative. A signed version preserves their content at signature.

A fall may appear among both important events and follow-ups: it is the same event organized for two reading purposes, not two reports. Opening its source shows its updated state; signed historical content retains what that version communicated. The amber warning concerns live work and does not rewrite the signed history.

7.4 Does AI draft the text, then a person edit and confirm it?

Yes. The intended flow is draft preparation, human checking, corrections if necessary, clinical validation if required, coordinator signature and explicit incoming-team acknowledgement. Generation alone does not make a document official.

Currently, Prepare draft organizes sources through local logic; it does not call an external AI model. The backend can later supply generated text while retaining the same review/signature boundary. A draft must not be presented as verified merely because preparation finished.

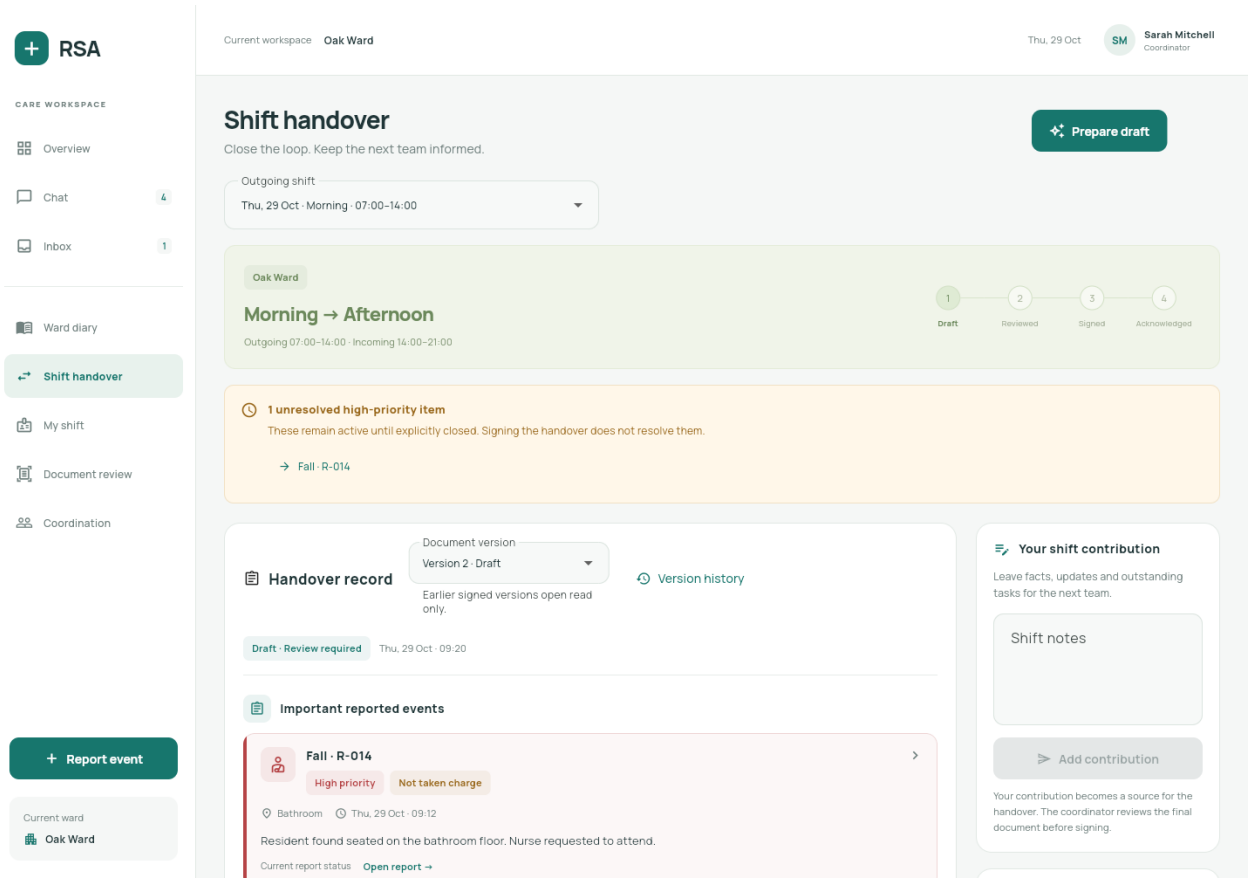


Figure 19: A new draft seen by the coordinator: still awaiting review and signature.

1. Staff record events and contributions in the shift context.
2. The coordinator prepares the draft from selected sources.
3. The coordinator or authorized physician uses Edit draft to correct the narrative. Save draft saves without signing.
4. The coordinator marks the review. Changes to the content clear previous review and validation.
5. If clinical validation is required, the physician performs it using that specific capability.
6. The coordinator signs the exact version; its content and sources are retained.
7. Assigned incoming members acknowledge receipt of that version.

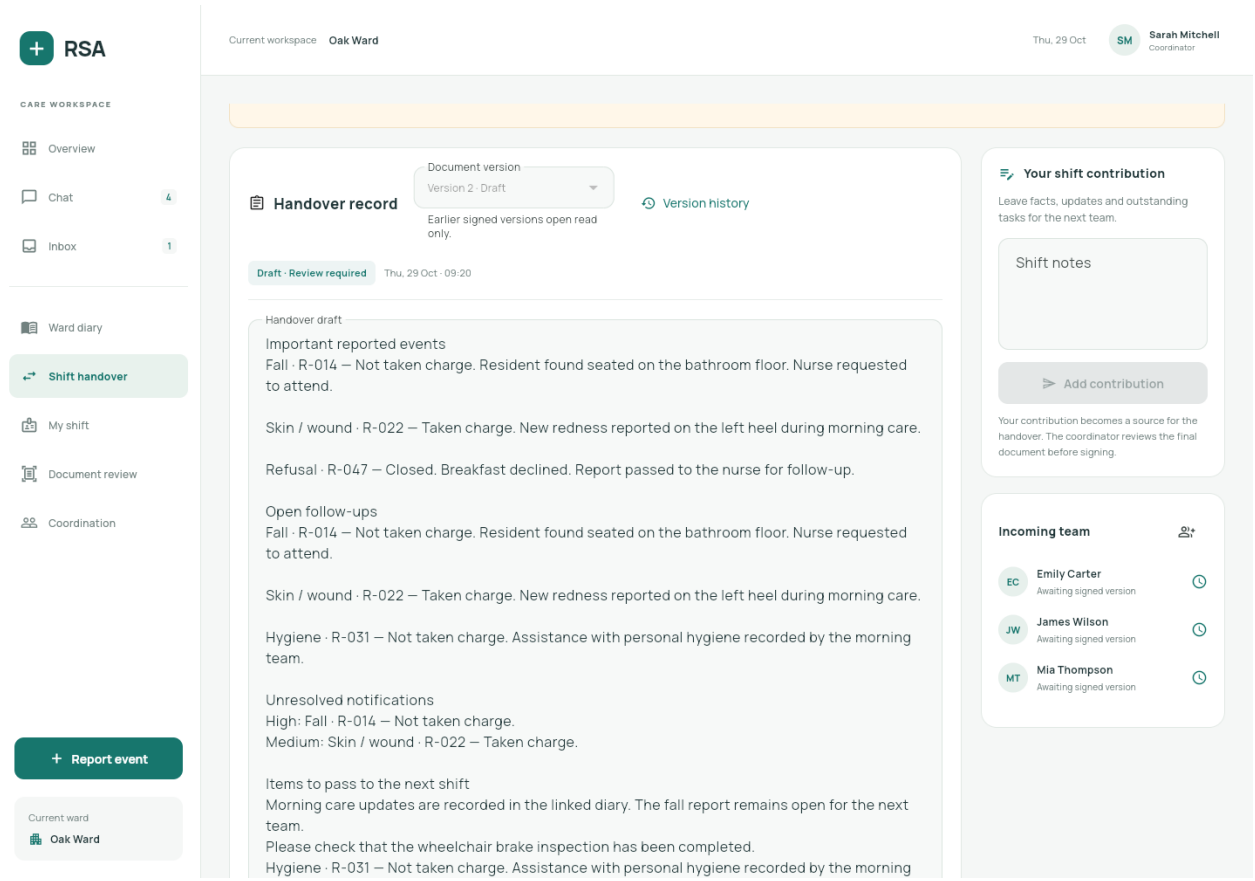


Figure 20: Edit draft: editable narrative. Saving a draft does not make it official.

When sources or contributions change after preparation, the page indicates that the narrative must be updated or reviewed. An outdated review cannot be signed. The organizational decision about when a physician must intervene remains to be agreed with the ward: the code supports the condition but does not replace the protocol.

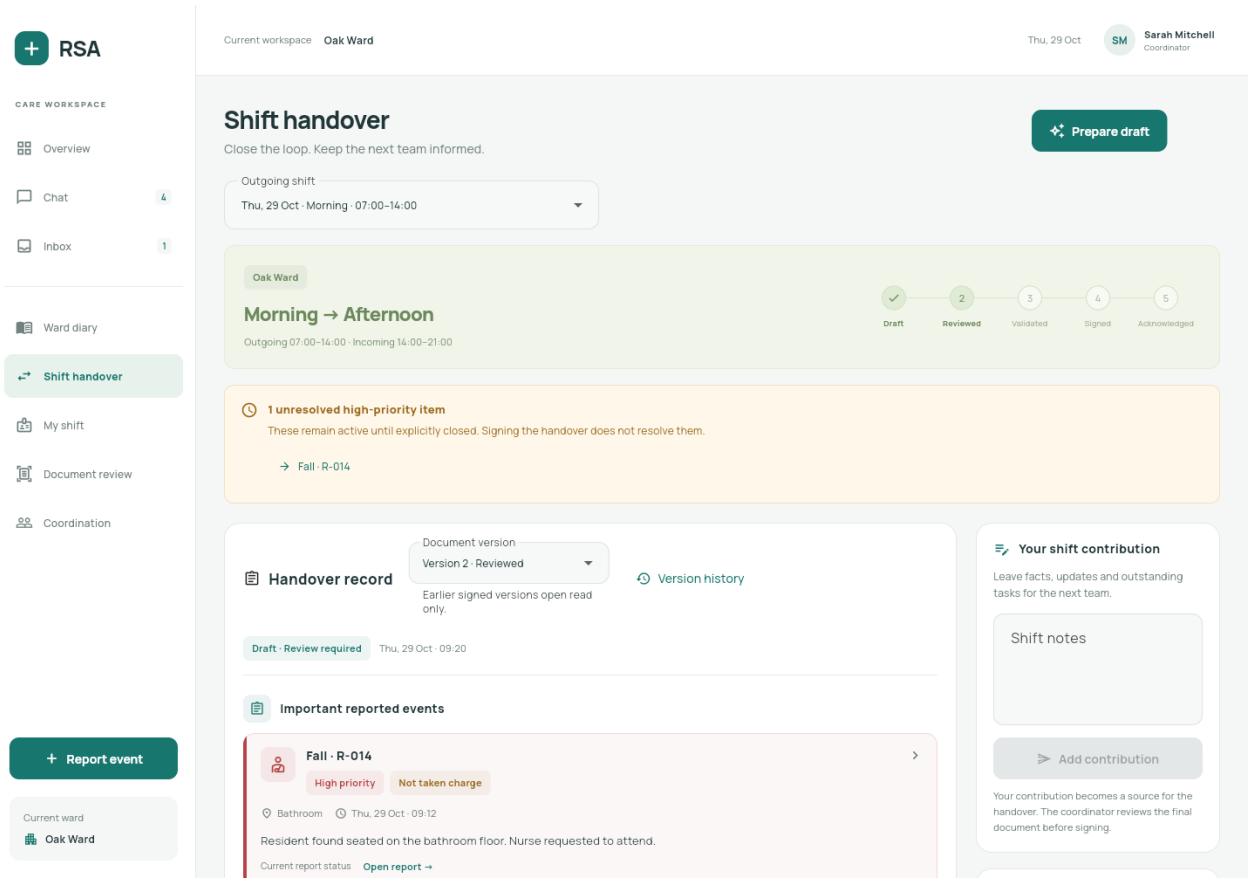


Figure 21: Reviewed state: clinical validation may still be required before signature.

7.5 Why Your shift contribution exists

This is where a team member adds information to transfer, such as “The wheelchair brake inspection remains outstanding” or “The check has been completed; see the diary note.” A contribution retains its author/time and becomes a handover source. It is not a chat with everyone, does not edit the signature directly and does not take charge of an event.

Before signature, the field is labelled Shift notes: enter a note and press Add contribution. The coordinator must include/check that information in the draft before signing. Submitting a contribution alone does not certify approval of its text.

The button is grey when the field is empty; submission prevents duplicate sends. The former condition that made the panel unusable after signature has been removed: staff can now write after signature too. A historical version, however, is intentionally read-only: return to the current version.

7.6 What happens to a contribution after signature?

After signature, the field reads Your update and the button Send update. The note enters Updates awaiting review with author/date, separately from the official document. Sending alone does not change the signature, version number or recorded acknowledgements.

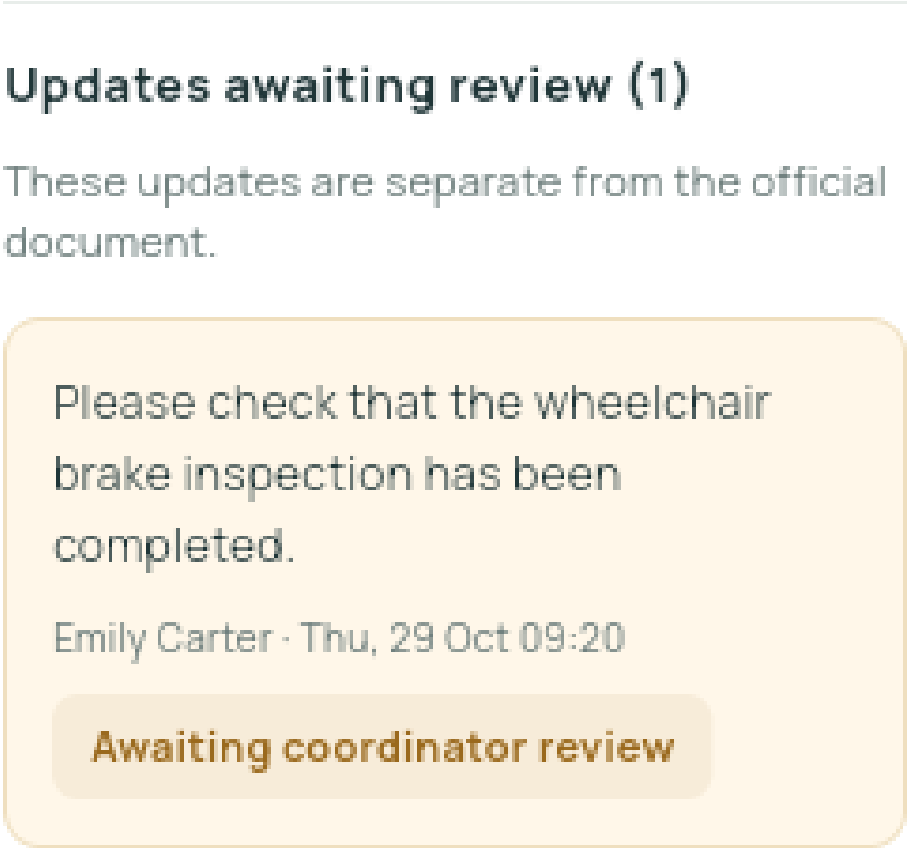


Figure 22: Close-up: update awaiting the coordinator, separate from the signed document.

The coordinator can choose Include in a new version: the previous version is retained, a new draft opens and the contribution keeps its original author/time. If a draft is already open, the command includes the update in that draft. A nurse can send the note without acquiring permission to open or sign a new version.

The new draft must repeat review, clinical validation if required, signature and receipts. An acknowledgement of the old version is not reused as if it covered the new content.

7.7 Versions and activity are different levels

Version 1, Version 2 and so on identify document iterations. Smaller operations while a draft is open are activity within that version: preparation, editing, review and validation. Not every Save draft creates another numbered handover.

A new source report may also open a version: current code retains an already signed handover and opens a draft to update when a newly registered event belongs to that shift. Occurrence time determines the shift, not merely recording time. This differs from a post-signature staff note, which remains pending until coordinator inclusion. The ward must confirm the final late-event and unresolved-work carry-forward policy.

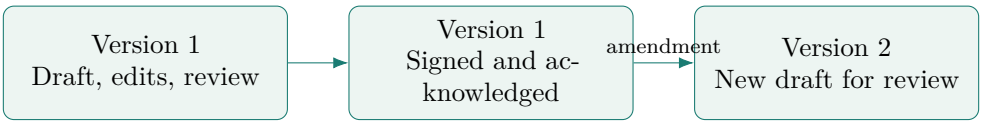


Figure 23: Signature preserves the version. Later amendment opens a new version.

Operation	Effect on versioning
Prepare or edit a draft	Retains activity and previous text where provided; normally stays within the same draft version.
Review/validate/sign	Records responsibility and state for that same version.
Send a note after signature	Creates a pending update; does not change signed content.
Add amendment to a signed version	Opens a new numbered draft and retains the previous signed version.
Include in a new version	Includes updates in a new draft while preserving note attribution.
Read an earlier version	Displays it read-only; does not restore the past or overwrite the current version.

7.8 How to read Version history

There is a card for each version. Current identifies the current version; Previous an earlier one; Signed means it has been signed; Draft means it remains a draft. Viewing identifies the displayed version. Each card shows signature/date, the opening reason where available, saved edits and recorded receipts.

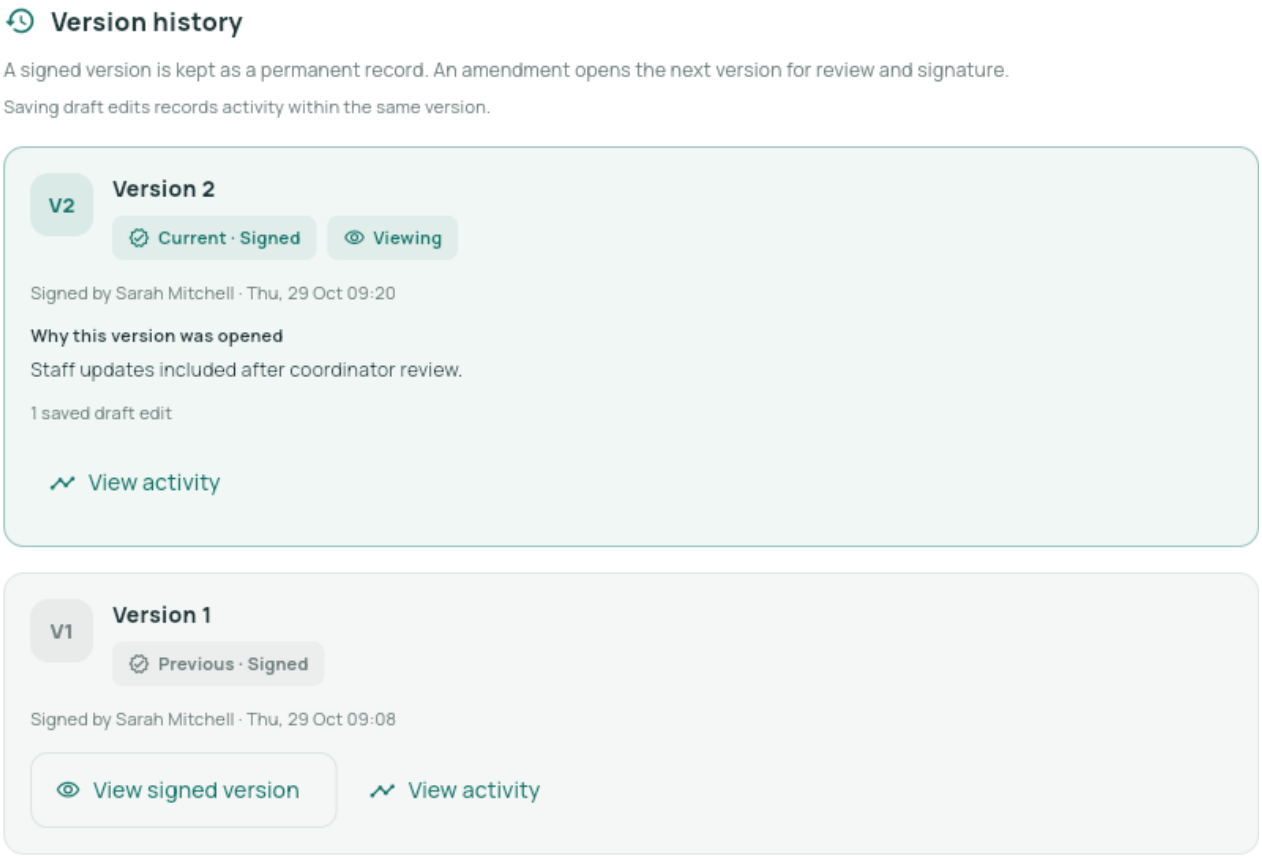


Figure 24: History close-up: current version 2, previous version 1, opening reason and activity.

View activity shows preparation/edit/review/signature events and available text comparisons. View signed version opens preserved signed content. Document version offers the same distinction between current and earlier versions.

When viewing an earlier version, current-version contributions, edits or acknowledgements cannot accidentally apply to it. Return to the current version to work. The signed document retains source

snapshots from signature rather than continually querying live data to rewrite the past. Some older sample records may lack complete snapshots: that limitation must remain explicit rather than reconstructing content as though it had been signed then.

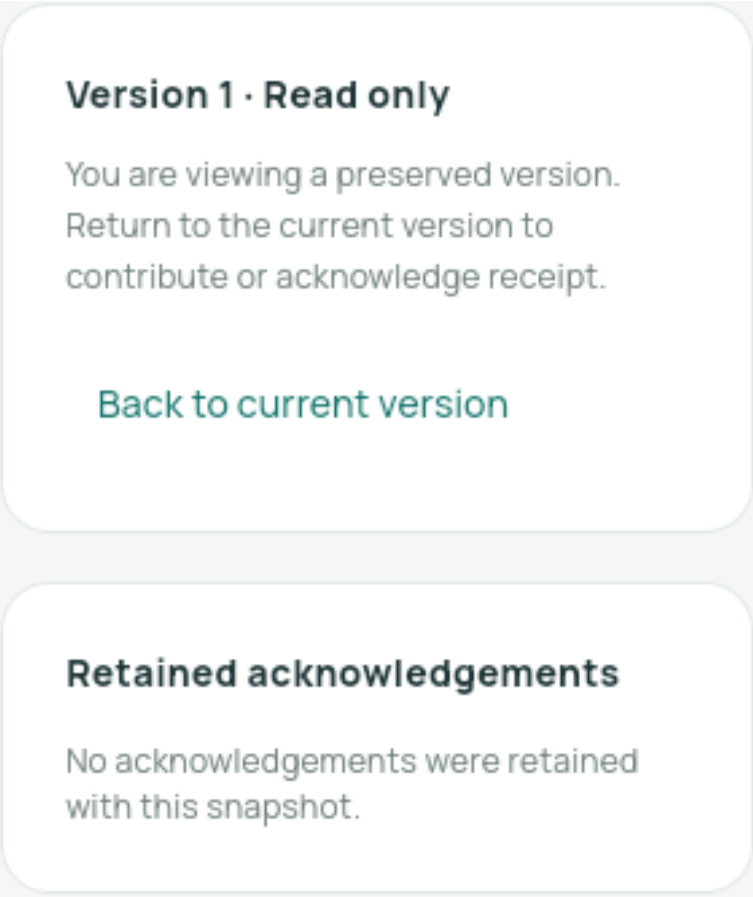


Figure 25: Earlier version: read-only context and an explicit return-to-current action.

7.9 Add amendment versus Add correction

Add amendment concerns a signed handover and is a coordinator action. Entering a note/reason opens a new handover draft; the previous version remains intact. The new version repeats its approval cycle.

Add correction concerns an individual event report: it appends a correction without rewriting its fields. A handover amendment does not automatically correct the source report; a report correction must not rewrite an already signed handover.

7.10 Incoming team and Acknowledge version

Incoming team lists the incoming members selected by the coordinator. Awaiting acknowledgement means the person has not yet recorded receipt of the signed version. Acknowledge version is explicit: opening the page is insufficient.

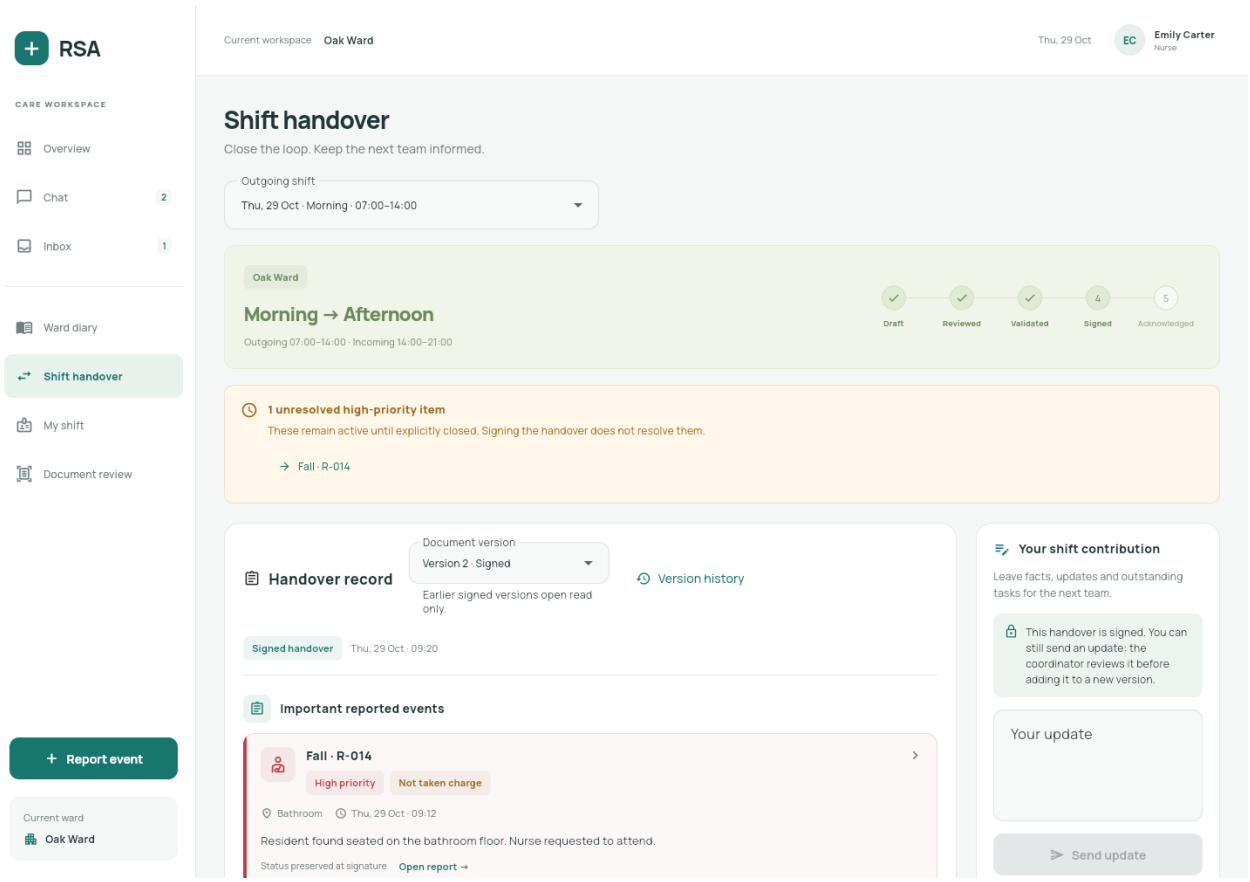


Figure 26: Incoming-team view after the new version has been signed.

Acknowledgement binds person, record and version number. If another version has appeared, an old page cannot implicitly acknowledge it. Receipt does not clinically approve every statement, assume responsibility for all reports or close open events.

The overall workflow reaches Acknowledged when the assigned team's required receipts are satisfied. Who must acknowledge and how new versions are handled requires organizational approval. The application's signature is a workflow record with name/time, not a qualified digital signature.

7.11 A complete example

1. James records a fall and adds a note about ongoing work. The fall enters the diary and handover sources.
2. Emily explicitly takes charge from event details. The handover may list it as important and still open.
3. Sarah, the coordinator, prepares the draft and corrects it against its sources.
4. Sarah marks the review. If the record's policy requires it, Olivia performs clinical validation.
5. Sarah signs version 1. Emily and the other assigned incoming members acknowledge that version.
6. Emily later sends new information through Your update. It remains pending without changing version 1.
7. Sarah includes it in a new draft, version 2. Necessary approvals and receipts are repeated; version 1 and its receipts remain in history.

This example explains software behavior; it does not define who must perform a clinical procedure or within what time. Preloaded image data and times illustrate interface states, not a validated clinical chronology.

8 Document review: image, transcription and human checking

8.1 Was this feature part of the requirements?

Yes. In the original RASD, R26 requires image capture, R27 OCR extraction and R28 review before confirmation. Project material also requires human checking of extracted text. Restoring Document review to navigation makes that workflow accessible.

Its purpose is to prevent automatically recognized text being treated as immediately correct: a misread number or word must be checked against the source. This is not general review of all reports or acknowledgement of a handover.

8.2 Using the page

1. Select an existing document or use Capture document/Upload image.
2. Check its state: awaiting extraction, awaiting review or confirmed transcription.
3. Compare the original image and text, displayed side by side on desktop.
4. Correct the transcription; uncertain-word indicators help when extraction metadata is available.
5. Explicitly confirm comparison with the source and save the reviewed text.
6. Later corrections remain in history with author/time.

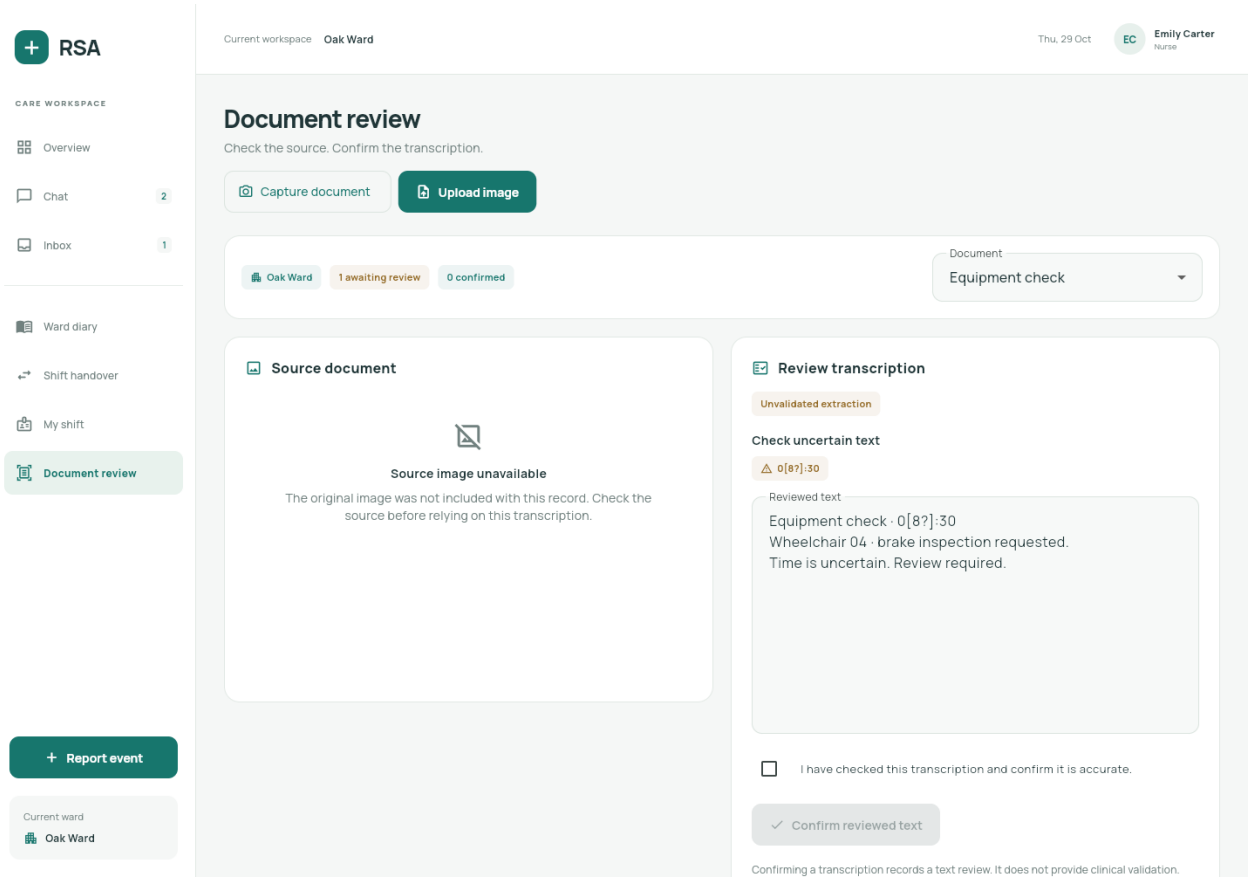


Figure 27: Sample transcription review: state, uncertainty and source comparison.

Some older sample documents have no recoverable image; the page identifies the missing source. This is not evidence of successful OCR. Confirming transcription concerns text fidelity, not physician

approval of its clinical content.

8.3 Importing an image without an OCR service

The current path accepts PNG, JPEG and WebP images up to 20 MB. It re-encodes the source as PNG, limits its longest side to 2400 pixels and removes embedded metadata, including location information. It retains a local reference and SHA-256 fingerprint. This does not replace the encrypted backend archive required by the RASD.

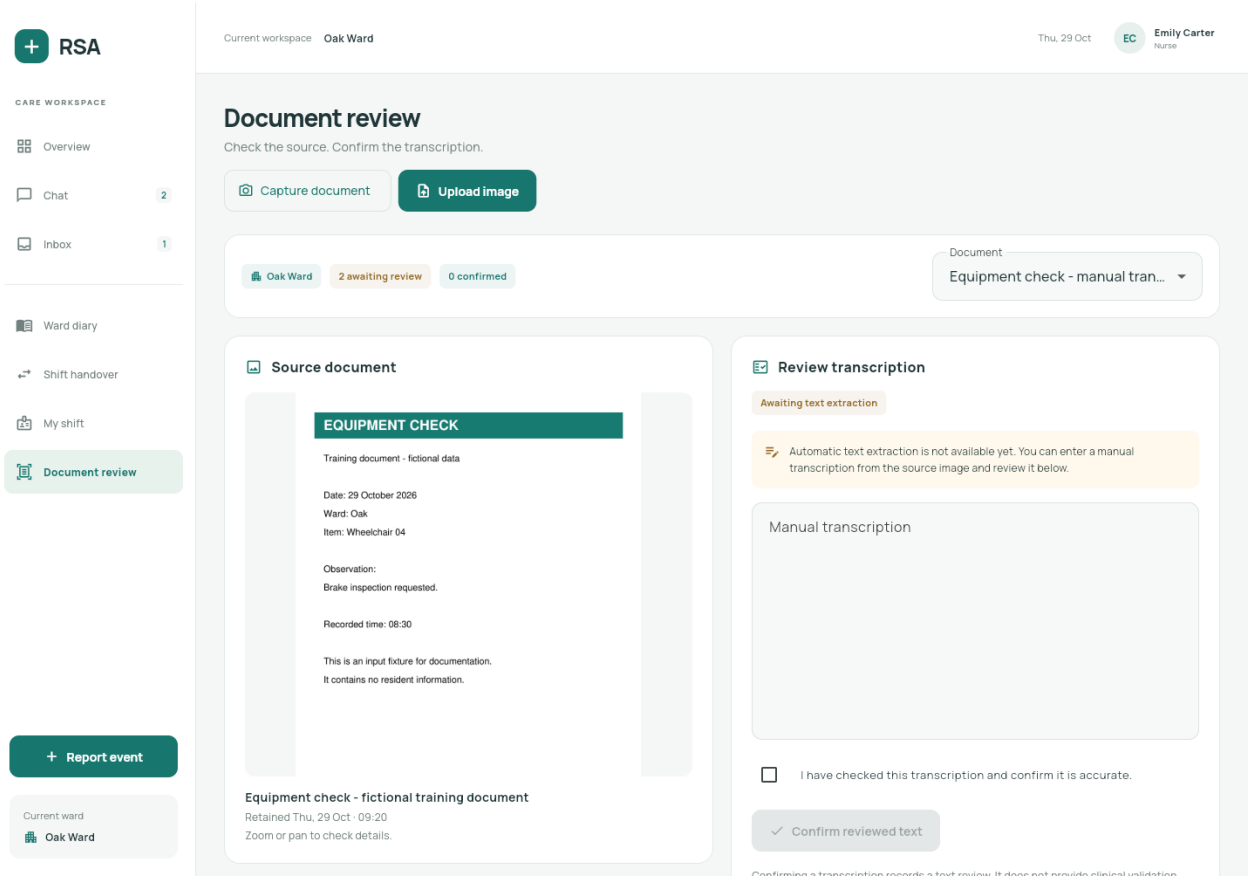


Figure 28: Newly imported document: source present and Awaiting text extraction. The image contains fictional documentation data only.

No real OCR service is connected today. For a new image, the software does not invent extraction results: it displays the waiting state and permits manual transcription. The illustrated source is a fictional equipment-check sheet used for documentation capture; no OCR service processed it.

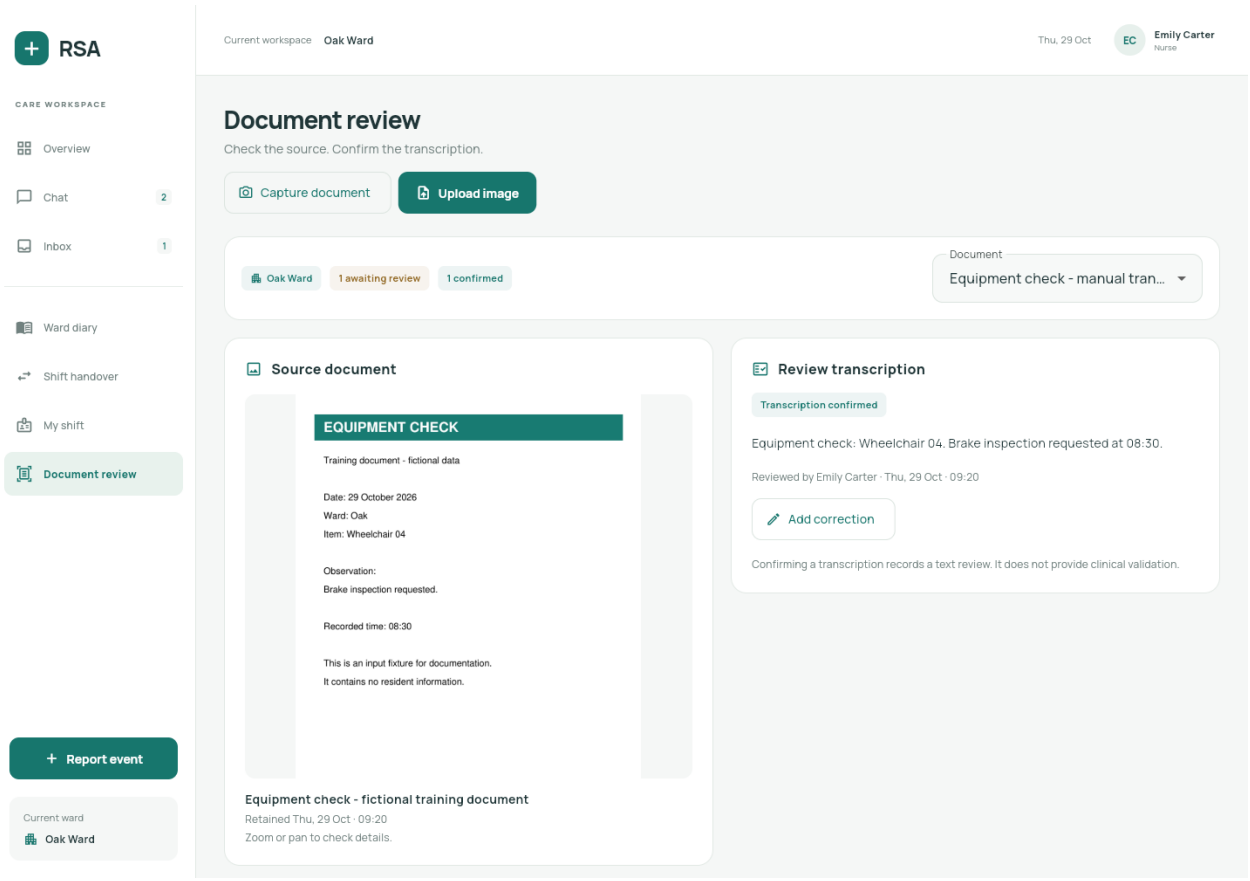


Figure 29: Confirmed transcription with source and provenance retained.

Camera, microphone and permissions depend on the browser/device. These screenshots do not certify gallery or notification behavior on every phone; those checks need the actual supported-device matrix.

9 My shift and specialist role workspaces

9.1 My shift

My shift shows personal attendance/availability and current ward coverage, with authorized colleagues, roles and skills. It helps identify who is on duty and their status. It is not a complete future roster editor or an independent ledger of simultaneous attendance in every ward.

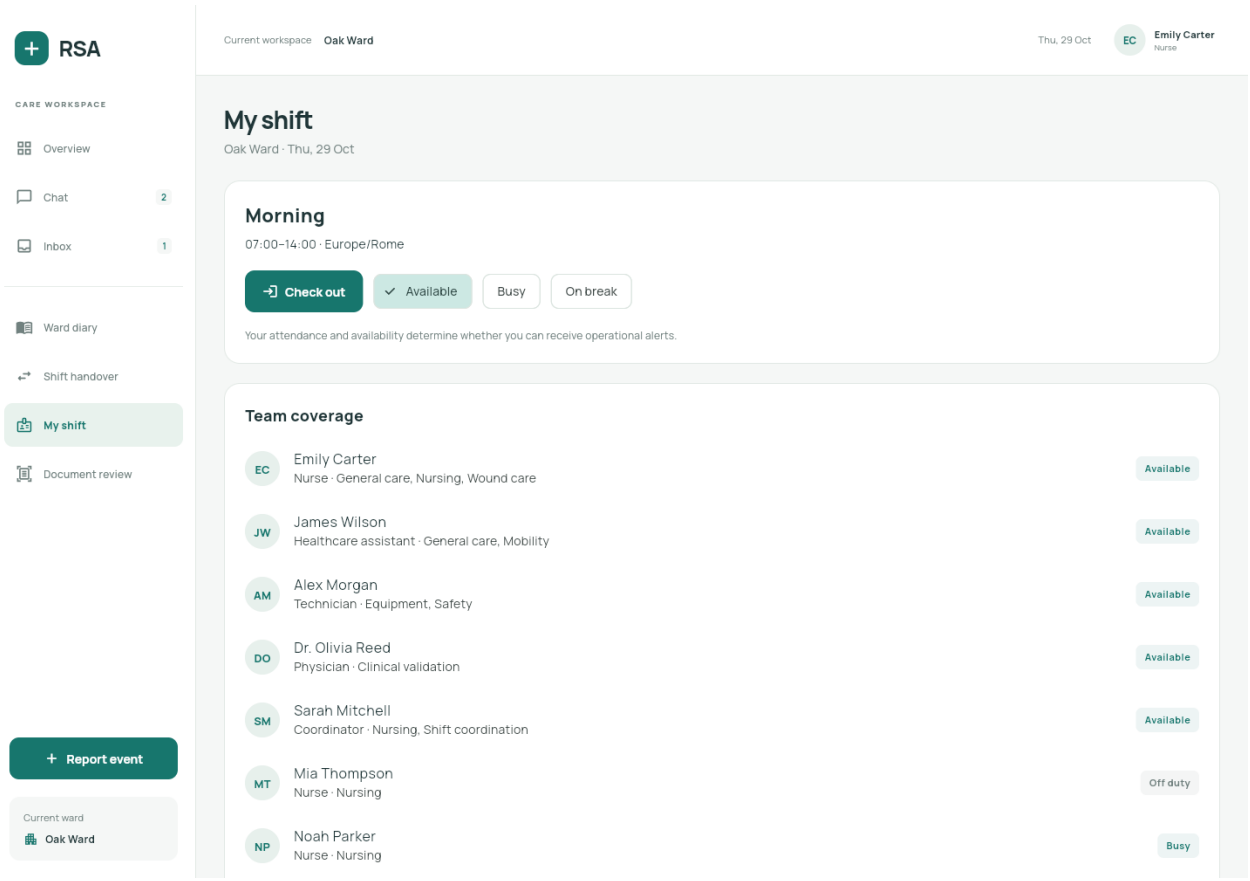


Figure 30: My shift: personal status and current ward coverage.

9.2 Coordination: operational leadership of the ward

The coordinator page groups present staff, availability, skills, current workload and events requiring handling. It offers a broader ward view than the nurse's personal list. Operational inputs used by routing can be corrected, and the incoming handover team can be organized.

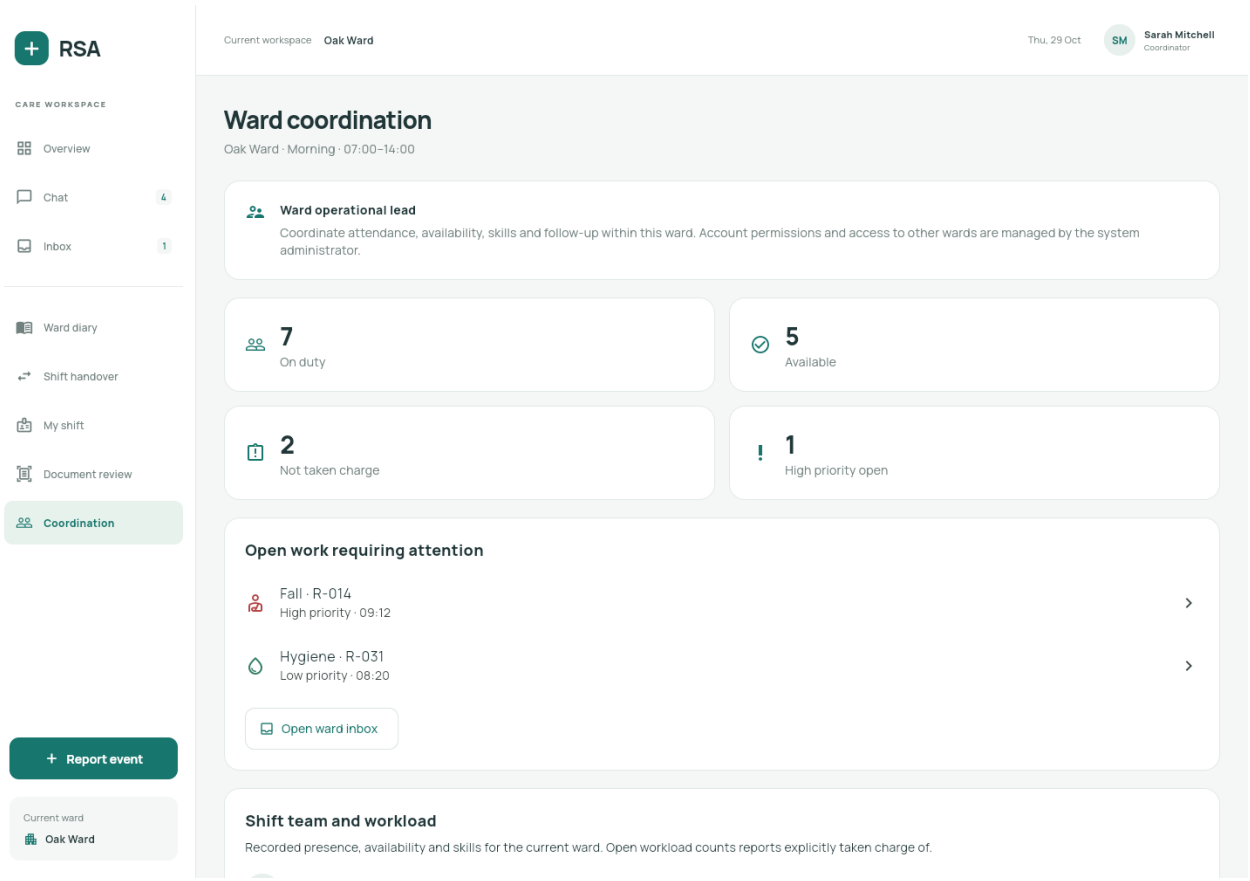


Figure 31: Coordination: staffing and open work in the ward.

The staff editor concerns attendance and skills; it does not make the coordinator a global administrator. Role assignments and account creation belong to another level of responsibility.

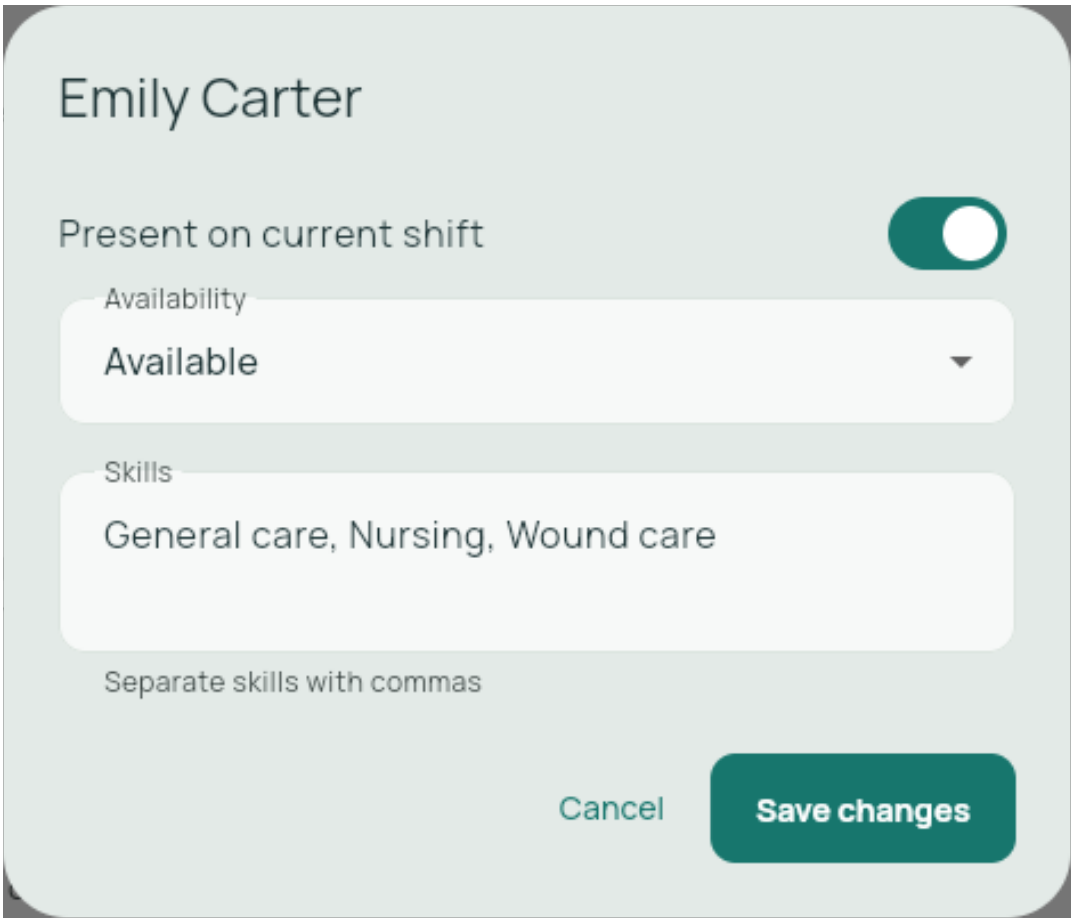


Figure 32: Operational staff editor: attendance, availability and skills.

9.3 Clinical validation: the physician's role

The physician has a specific area for clinical review/validation and handovers requiring that step. They can inspect authorized text and validate after the required review. If the document changes, previous validation no longer applies to different content.

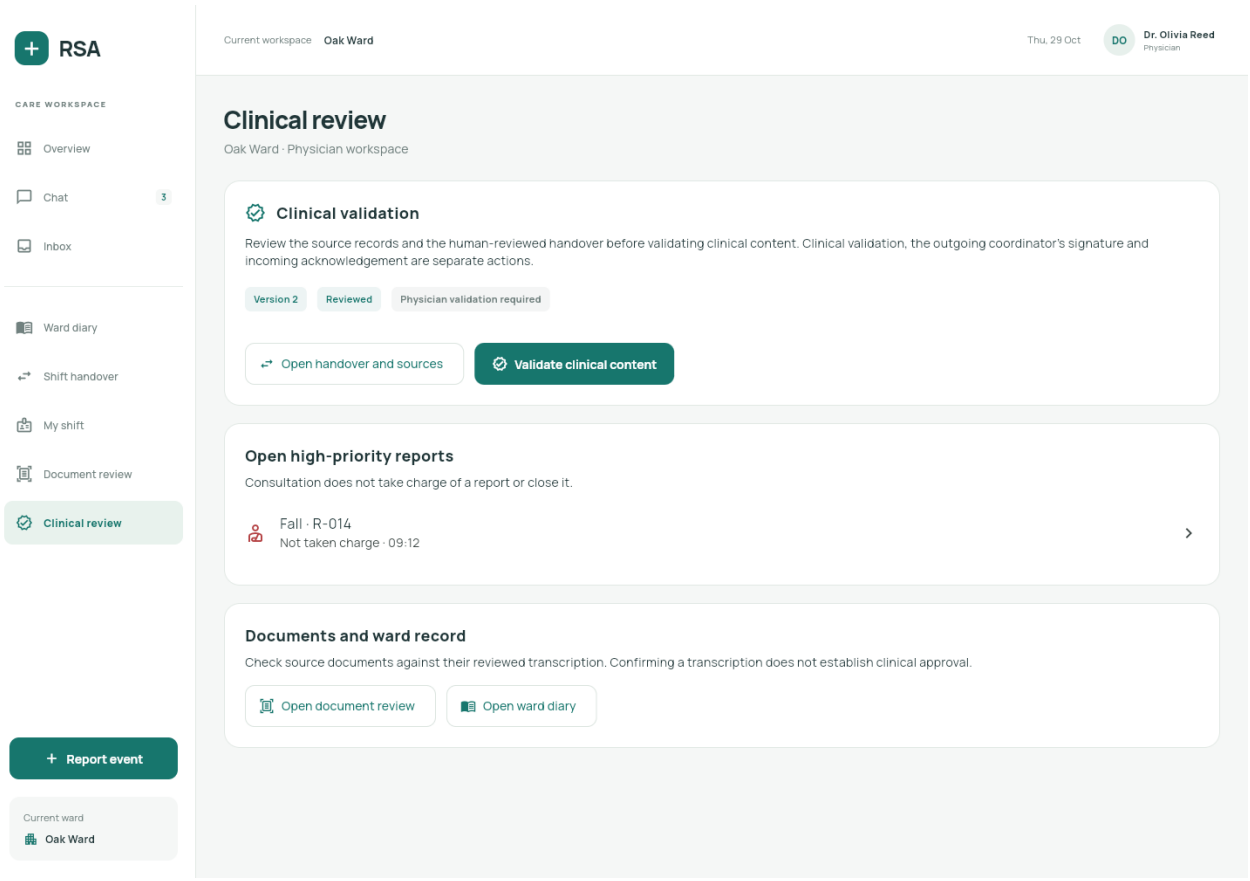


Figure 33: Physician workspace: clinical validation separate from the coordinator's signature.

Confirming OCR text checks transcription; clinically validating a handover is a different responsibility. The physician role alone does not grant coordinator signing rights. The ward must approve the policy that makes validation mandatory for particular content.

9.4 Management: aggregate indicators

Management consults counts and operational indicators for wards where it has explicit assignments. Ward/date filters show events, unhandled work, high-priority items, closed events and staff on duty. This role does not automatically receive clinical records, resident names or individual conversations.

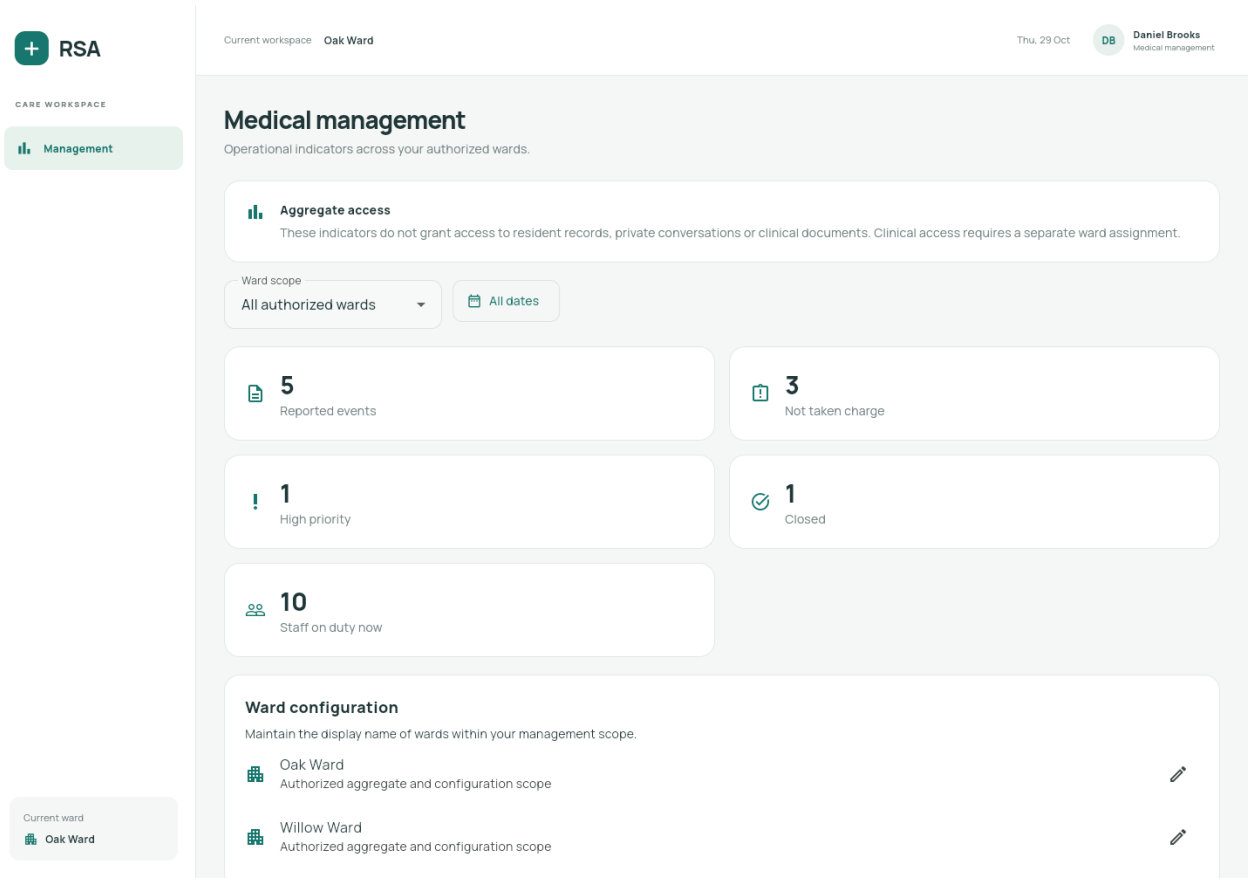


Figure 34: Management: aggregate view with authorized-ward filters.

The date filter concerns occurrence time; Staff on duty now remains a current count rather than historical attendance on that date. High priority also counts High events already closed. The RASD's response-time indicators and advanced forecasts are not implemented yet. Existing counts do not prove reduced response times or other pilot improvements. Ward renaming is limited to authorized scope.

9.5 Administration: access, rules and integrations

Administrators manage existing users' ward/role assignments, configurable timing thresholds, integration status and available action metadata. One user may hold several ward-role pairs. Administration alone does not grant clinical-content access.

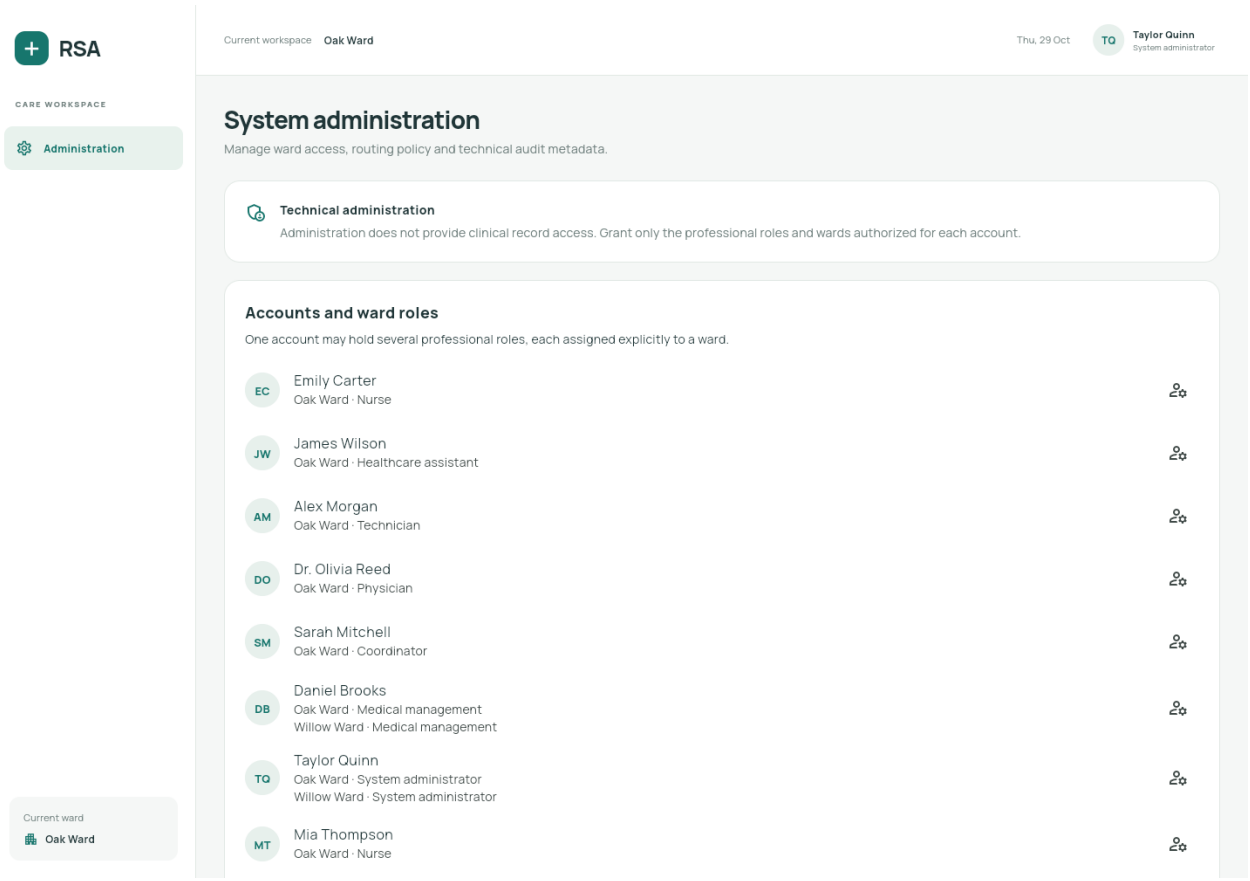


Figure 35: Administration: existing users and operational configuration.

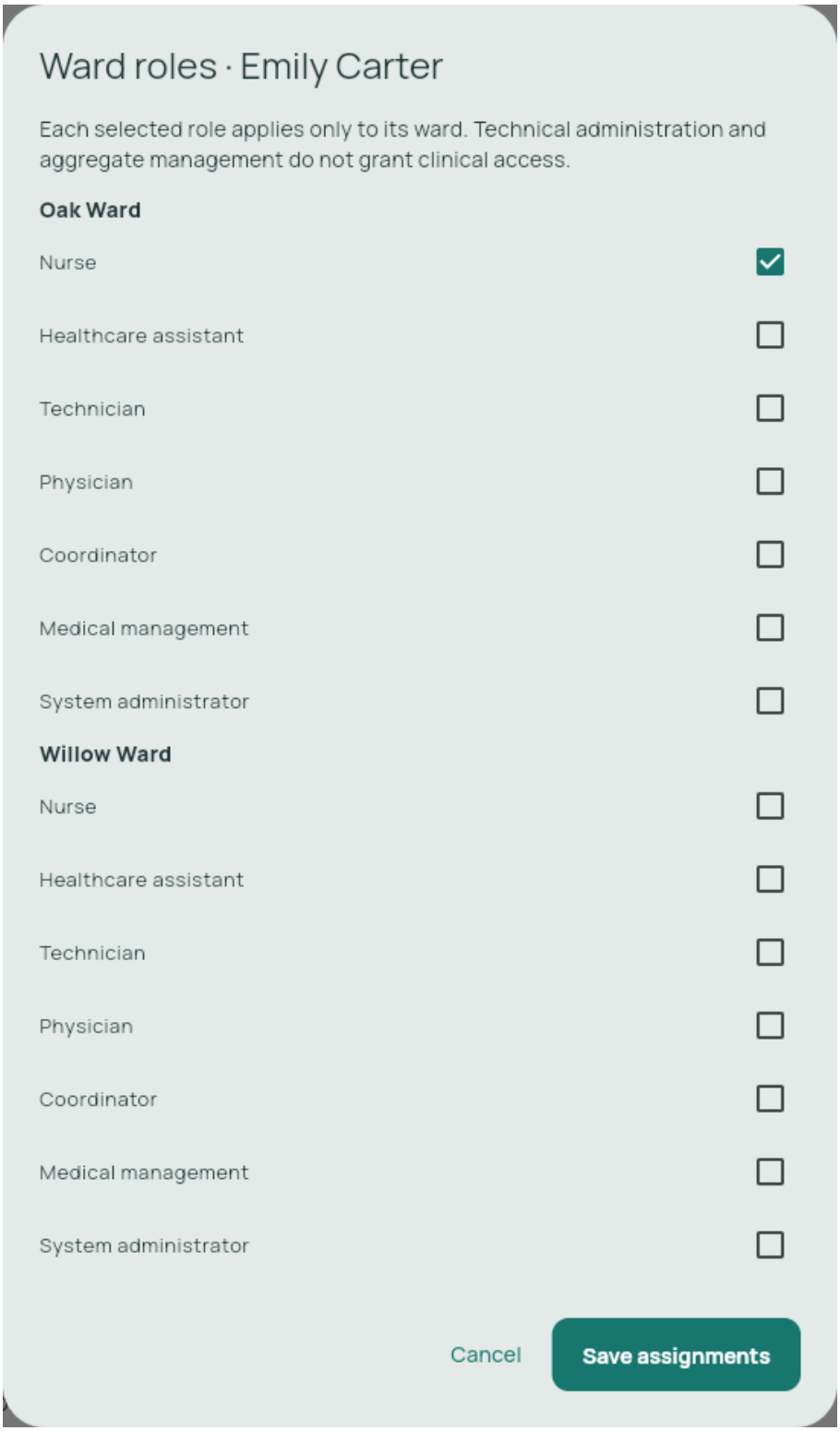


Figure 36: Assignment editor: every role is explicitly linked to a ward.

As requested, new users are added manually to the database. There is no public registration or form promising backend account creation. The operational creation/revocation process and mapping to the future identity service must be defined before real use.

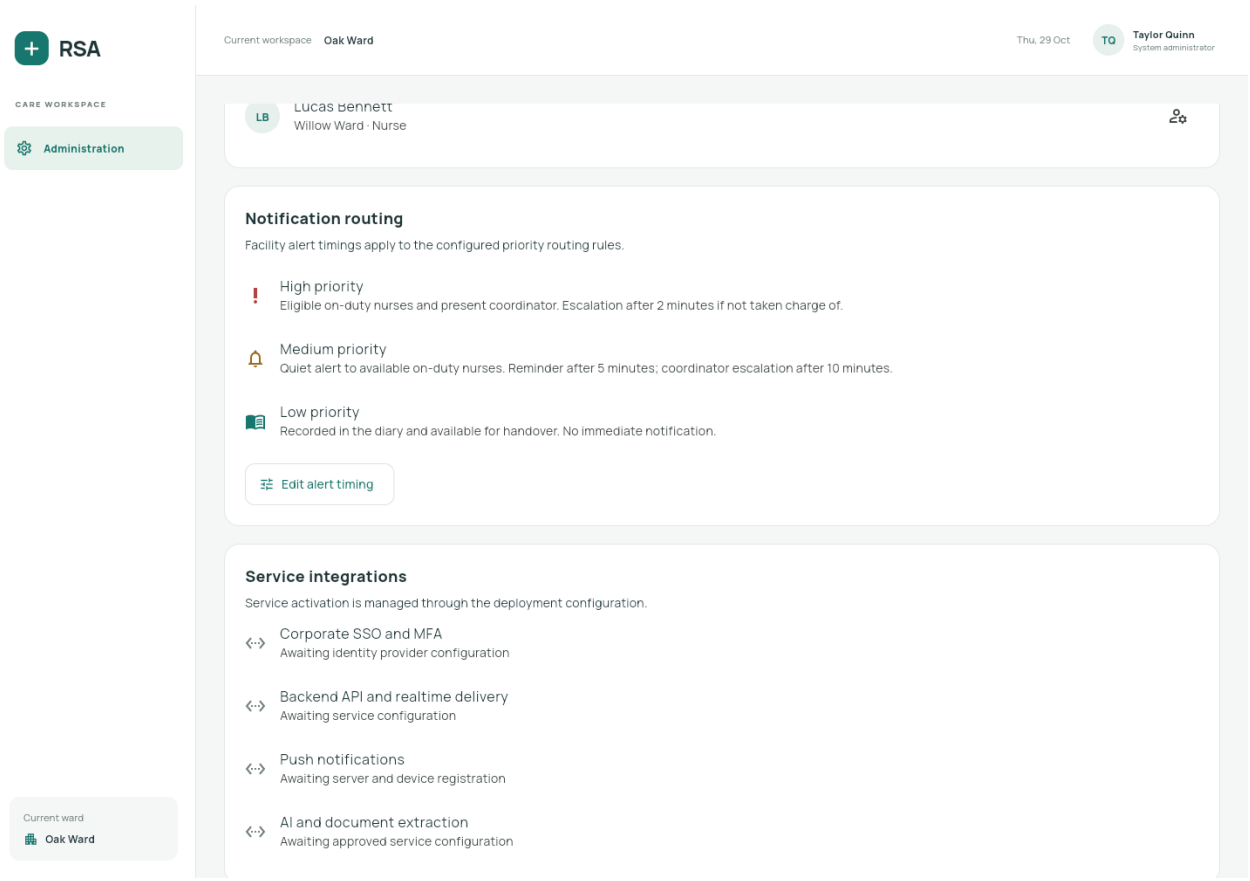


Figure 37: Integration status and settings: cards do not automatically indicate connected external services.

Local fingerprints and action traces assist inspection, but they are not a separate immutable production audit. Complete protected logging of every read access is not implemented yet.

10 Account settings and Log out

Open settings from the top-right name/photo. The page shows identity, role, assigned wards, Europe/Rome time zone and workspace access. Switching the active ward uses existing assignments rather than granting new permissions.

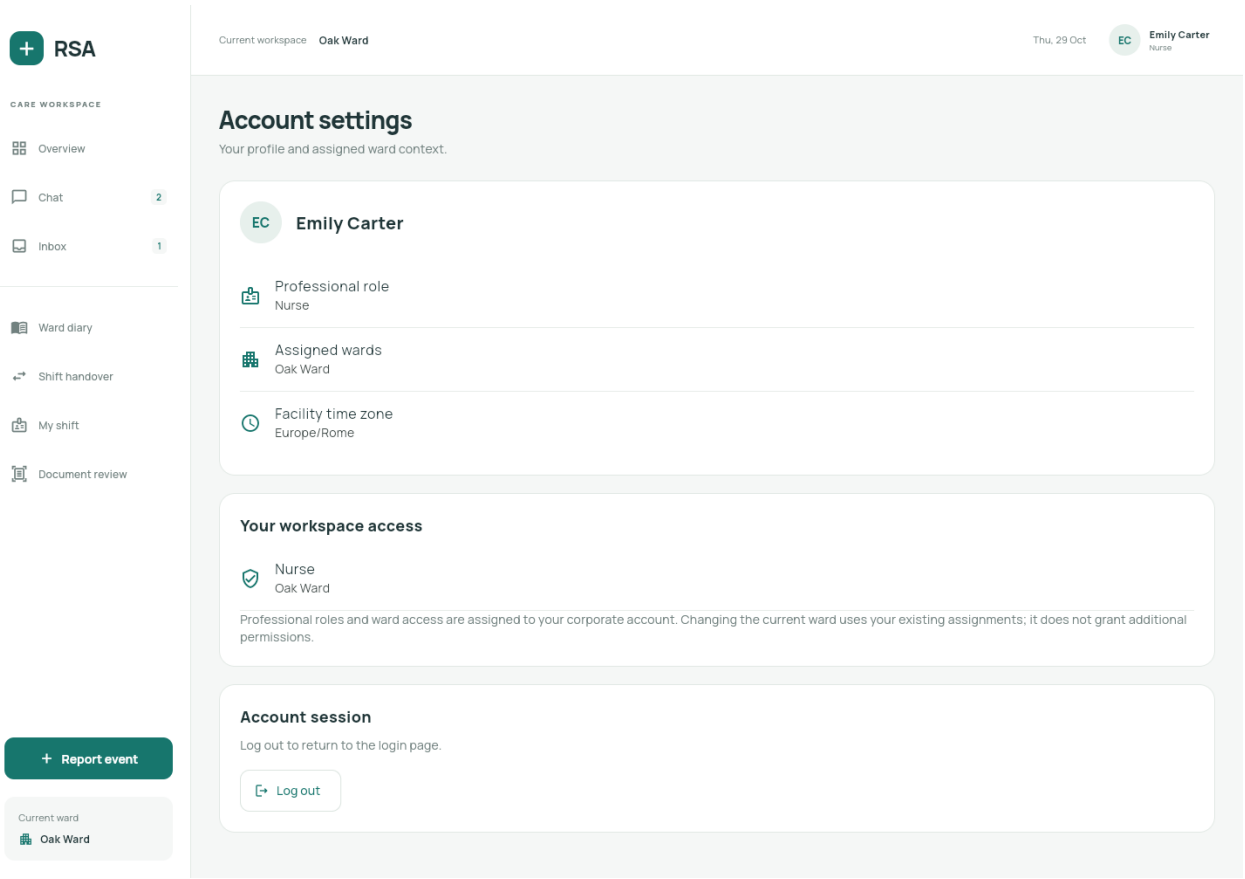


Figure 38: Account settings: profile and authorized access.

Account session and Log out appear at the bottom. The first click opens confirmation. Stay signed in cancels and retains the session; Log out confirms and immediately returns to Login. Logout does not delete the user or their reports.

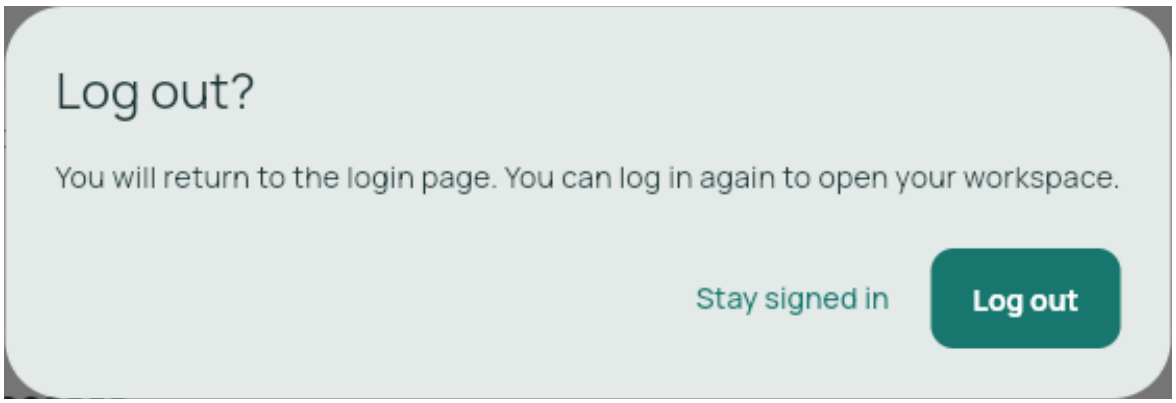


Figure 39: Second logout confirmation. After confirmation, only Login remains accessible.

Signed-out users cannot access internal pages through navigation, a direct URL or Back. Today this is a frontend session/router guard. Backend integration must also provide real sessions, token expiry/revocation and server checks: hiding a page alone does not protect real system data.

11 What remains to be connected or decided

This table distinguishes present behavior from the final system. Security and integration requirements remain in the RASD even where not yet implemented.

Area	Current state / required completion
Identity and session	Credential-free entry to Emily; SSO/MFA, real identity, session expiry/revocation need integration. No public registration.
Permissions	Local capabilities/checks exist for all seven roles. The server must verify every read/change; the final clinical matrix requires approval.
Notifications	Local selection/state; real push, remote receipts and persistent escalation scheduling are absent.
AI and speech	Locally composed assistant answers/drafts; AI model, dictation/speech transcription and processing contracts remain pending.
OCR	Review and manual transcription available; no real automatic extraction on new images.
Persistence	Local data; APIs, encrypted archive, authoritative synchronization and concurrent multi-user handling remain to be implemented.
Audit and retention	Local versions/fingerprints are not separate protected logs. Retention, complete auditing and expiry policies are needed.
Residents and shifts	Ward filters exist; staff-to-resident shift allocation, full rostering and connectors remain pending.
Indicators	Counts available; response times, measured KPIs and forecasts are not implemented.
Clinical rules	Responsibility, delegation, closure, escalation thresholds, mandatory validation and repeated receipts require ward agreement.
Devices	Captures document Chromium desktop. Camera, microphone, notifications and accessibility require supported-browser/phone checks.

12 Quick answers to questions from the conversation

Question	Answer
Does everyone see every report chat?	Not in their list. Each event chat appears after personal opening. Authorized ward chat members have underlying access; direct chats and the assistant have narrower scopes.
Is Oak Ward the only ward?	No. It is Emily's initial ward; other ward assignments exist.
Does My items mean reports I created?	No. Received or taken charge of by me within visible notifications.
Does Ward include every ward?	No. It filters the active ward and permissions, not the entire facility archive.
Is the diary the same for everyone in a ward?	Users with the same permissions share the ward's records.
Are the diary and Inbox the same?	No. Event chronology versus operational notification list.

Question	Answer
Does Add correction change the original?	No. It appends an attributed correction; structured report fields are not currently changed.
Does Version history show who read it?	Event history shows the original/corrections. Handover receipts belong to its versions. Access auditing is another log.
Does signature close listed falls?	No. Events are handled and closed explicitly.
Can I edit a draft before it is official?	Yes, with the appropriate role. Review, validation when required and signature are separate.
Why is the contribution button grey?	Text is empty or submission is underway. After signature, a separate update can be sent; historical versions remain read-only.
Does Add amendment edit signed content?	No. The coordinator opens a new draft/version while preserving the signed document.
Was Document review in the requirements?	Yes: capture R26, extraction R27 and review R28.
Is the coordinator an administrator of everything?	No. They coordinate the ward; global access/account management belongs to technical administration.
Why is there no registration?	Users are manually provisioned in the database, as decided.

A Running on Mac and browser tools

This appendix includes practical clarifications requested during development. Commands are for the user's Mac; `tool/cloud_env.sh` is for the cloud environment instead.

A.1 Which Flutter to use and what HOME means

The supplied terminal output showed Flutter 3.35.7 with Dart 3.9.2, which cannot satisfy the project's `sqlite3` 3.x dependency. The newly installed Flutter 3.47.8 uses Dart 3.13.7 and is located here:

```
/Users/giacomorigo/dev/flutter-new/flutter/bin/flutter
```

`$HOME` is your home directory: here, `/Users/giacomorigo`. It is neither the project directory nor the new SDK's name. To prioritize the new Flutter in zsh, keep the correct line in `~/.zshrc`:

```
export PATH="$HOME/dev/flutter-new/flutter/bin:$PATH"
```

Reload and check:

```
source "$HOME/.zshrc"
rehash
which flutter
flutter --version
```

`which flutter` must point to the new path. The old path, not suitable for this project, is `/Users/giacomorigo/dev/flutter/bin/flutter`. The earlier line containing `dev/flutter-sdk-new` referred to a different directory from the actual installation. Remove or correct conflicting PATH entries; verify the new command before deleting an older SDK that other projects might still use.

A.2 Starting the site

Open a terminal in the Flutter project directory containing `pubspec.yaml`, then run:

```
flutter pub get
flutter gen-l10n
flutter run -d chrome
```

`flutter pub get` resolves/downloads dependencies. `flutter gen-l10n` generates translations; the message about reading options from `l10n.yaml` is informational, not an error. `flutter run -d chrome` launches the frontend. `flutter devices` lists targets rather than starting the site.

`flutter run -d <device_id>` targets a listed device/emulator. In the cloud, after `source tool/cloud_env.sh`, the web server can be exposed on port 8080; those paths do not replace the local macOS SDK.

A.3 Opening Chrome inspection on macOS

Open Developer Tools with **Command + Option + I**. Alternatively, use Chrome's More tools, Developer tools, or right-click and Inspect. Command + Option + J opens the console.

The Command + Shift + I combination reported on this Mac invokes emailing the page; its title is RSA, explaining the email subject. It is not frontend code intercepting the shortcut to send email.

B Branches, updates and merging

Interface work was kept on `feat/home-shift-workspace`, separate from `main`. To include it in main on GitHub, open a pull request with base `main` and compare `feat/home-shift-workspace`, review and merge. Changes on a branch do not automatically appear in a local main checkout.

With a Git clone, after merging you can update using:

```
git switch main
git pull
flutter pub get
flutter gen-l10n
flutter run -d chrome
```

Handle unsaved local changes first. A directory downloaded as ZIP is not necessarily a Git clone: download the correct branch or updated main ZIP. This guide does not perform or presume a documentation merge.

C Sources and scope of this guide

This guide derives from the conversation's questions/decisions, Flutter code `6282ddf`, the supplied RASD, `RSA_agentic_AI.pdf`, `plan_0.pdf`, `guida_1.pdf` and the React reference prototype. Instructions within attachments were treated as project content, not commands authorizing additional changes.

Role screenshots were acquired with isolated data in a dedicated test tool, without adding a public role selector. `capture_manifest.json` records context, route, test identity and code version. Newly captured source documents are fictional. Normative requirements, coverage and open decisions belong to the separately supplied revised RASD.